



Annual Review Report 2016



**higher education
& training**

Department:
Higher Education and Training
REPUBLIC OF SOUTH AFRICA



**UNIVERSITIES
SOUTH AFRICA**



College Principals



Published by the Higher Education and Training HIV/AIDS Programme (HEAIDS)
April 2017

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Message:

Deputy Ministry of Higher Education and Training



“... campuses have their own health services and are well placed to secure the links between testing, treatment and viral suppression”

Mduduzi Manana
Deputy Minister of Education and Training

My term as Deputy Minister has given me a chance to develop a close understanding of the Higher Education and Training HIV/AIDS Programme (HEAIDS) and appreciate its relevance to the prospects of students even long beyond their graduation.

This annual review reflects how funding of the HEAIDS initiative has changed. Just two years ago the programme was funded predominantly by international funding agencies, whereas in 2016 about 68% of HEAIDS' direct funding derived from domestic public sector sources. In addition to this, the public health sector makes a major contribution to HEAIDS through the supply of condoms and various medical consumables, such as HIV test kits and medication.

This change speaks to the question of ownership of the HEAIDS programme. Personally I am proud to be associated with this undertaking and feel enriched every time I attend a campus activation or dialogue. Equally satisfying is the knowledge that membership fees paid by colleges and universities make up a share of HEAIDS' funding and strengthen the accountability of the programme to the institutions that constitute the sector.

These are critical times for global – and national – efforts to defeat the epidemic and the HEAIDS response needs to be underpinned by the highest level of cooperation.

In the middle of 2016, UNAIDS sounded a warning that HIV prevention had stalled globally in the five years between 2010 and 2015. We are no longer gaining ground in terms of reducing the number of new infections and there is a danger of earlier gains being reversed.

“The power of prevention is not being realised,” UNAIDS Executive Director Michel Sidibé said in July. “If there is a resurgence in new HIV infections now, the epidemic will become impossible to control. The world needs to take urgent and immediate action to close the prevention gap.”

Thanks to the strengthening of its peer education infrastructure, HEAIDS is in a stronger position now than at any previous point to expand its primary HIV prevention activities – creating awareness and building knowledge about HIV transmission, distributing condoms, and encouraging medical male circumcision. The empowerment of young women is a critical strategy for HIV prevention in southern Africa and HEAIDS is fortunate to be working with young women who have aspirations and hope in the future.

Although HEAIDS supports the UNAIDS 90-90-90 fast track strategy on treatment, the higher education sector is still a long way from ensuring that 90% of students and staff at institutions know their HIV status. For the last two years, we have tested about 150 000 people a year in a population of some 1.75 million students and staff members. The strategy of organising two activations on every campus annually is clearly the first step to improving our testing coverage and we all need to commit to it.

In addition, we need to ensure that sound referrals for antiretroviral treatment (ART) are made available for every person who tests HIV-positive. In many cases campuses have their own health services and are well placed to secure the links between testing, treatment and viral suppression. Let us ensure that every HIV positive student is a treated student.

We have been reminded very powerfully in recent years of the sacrifices students and their families make in order to pursue higher education. In many cases, the hopes and welfare of entire families rest upon a talented member getting a good education and a job that pays decently. It is their escape route from poverty to a more secure life. Our job is to take all reasonable measures to ensure that their dreams are not sabotaged by undiagnosed and untreated HIV infection among our students.

The HEAIDS programme has gained confidence and made great strides in recent years. It is to be congratulated especially on bringing universities and TVET colleges under one protective roof and expanding the programme's reach. The years ahead are likely to be especially challenging as the demand for expansion continues alongside a drive for stronger attention to the quality of interventions and the evaluation of outcomes. I have no doubt that the programme will rise to the challenge.



Mduduzi Manana
Deputy Minister of Higher Education and Training

Foreword:

Chairperson HEAIDS Advisory Board



“...the establishment of a task team to address GBV is an important step for the whole sector”

Max Price
Chairperson HEAIDS Advisory Board

Few people would disagree that 2016 was an extremely tough year for higher education. There is not much to be said for hard times but they do sometimes reveal the qualities of the people you work with routinely and perhaps take for granted.

It was gratifying to see how individuals who are critical to the progress of the Higher Education and Training HIV/AIDS Programme (HEAIDS) continued to devote attention to the programme even, in some cases, when their campuses were in turmoil.

I think we have come to appreciate more deeply how much equity – not only in accessing higher education but also in deriving the full benefit from it – depends on the education system’s ability to deal with social issues that impact on students in different ways. It is necessary to ensure relevant measures are in place to level the playing field.

Individual institutions have their own ways of approaching this and some interventions are more focused and better resourced than others. But HEAIDS is possibly the only programme that seeks to address a range of health and social issues across the sector nationally.

Because of this HEAIDS could become part of the solution to some of the issues students are raising. The potential for this was highlighted by the issue of sexual assault and gender-based violence on and around campuses. This has long been a simmering issue at some institutions but in 2016 it began to boil furiously.

Clearly many institutions had under-estimated the extensiveness of the problem and the depth of feeling about it. Perhaps there was a tendency to treat rape and gender-based violence primarily as legal and disciplinary matters and, where possible, a question of providing emergency healthcare and counselling to survivors of assault. But the reality is that sexual assault and gender-based violence have strong social dimensions.

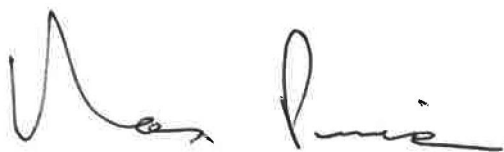
It was entirely appropriate therefore for HEAIDS to step up to the plate and offer to coordinate a national policy framework to prevent and respond to sexual assault and gender-based violence in the higher education sector.

Not only does HEAIDS have a vested interest (because these forms of violence accelerate the spread of HIV), but the programme is accustomed to working in the social terrain and bringing into the light experiences and issues that frequently dwell in the shadows of prejudice, shame and stigma.

In addition, by creating a national task team to develop the sectoral framework, HEAIDS is able to pool the expertise of many experienced individuals in our universities and colleges and from important organisations outside the sector. We look forward to receiving the results of the task team's work in the course of 2017.

During 2016 the emphasis of HEAIDS' work was sustaining the progress it had made in terms of student awareness and health-seeking behaviour and the integration of HIV content into diverse academic courses. Considerable attention and energy were devoted to strengthening platforms for delivery both in HEAIDS itself and in colleges and universities.

I have no doubt that these innovations will bear fruit in years to come and would like to assure Dr Ahluwalia and his team of our fullest support for their unfolding programme.

A handwritten signature in black ink, appearing to read 'Max Price', with a stylized, cursive script.

Max Price
Chairperson
HEAIDS Advisory Board

Message:

Deputy Chairperson



“The First Things First programme empowers our students to take responsibility for their health”

Helen Ntlatleng

Deputy Chairperson HEAIDS Advisory Board

The Higher Education and Training HIV/AIDS Programme (HEAIDS) gains greater understanding of the technical and vocational education and training (TVET) sector every year and continues to demonstrate its relevance to our students and staff.

We sometimes forget that young people do not get handed an automatic pass to adulthood along with their matric certificate. Yet they enter a world of greater freedom, opportunity and risk almost as they leave school. A programme like HEAIDS, which deals with the social complexities that students face, fills a very important gap.

Through First Things First activations we have come to appreciate that the risk of HIV varies among TVET colleges and even among different campuses of a single college. The number of students and staff members testing positive tells us that the risk is high in some areas, but lower in others.

Thankfully, South Africa has adopted the test-and-treat approach so those who are living with the virus can be offered antiretroviral treatment immediately after they learn their status. This empowers individuals to take responsibility for their health and protects them from the utter hopelessness that used to accompany an HIV-positive test result. It also reduces the chance of passing on the virus to someone else.

I would like to say how grateful we in the TVET sector are to those provincial health departments and district health services that have made the programme more viable in our colleges. Through the supply of condoms, various health consumables and – most importantly – skilled health workers on campuses you have made a huge difference. We would love to see this deployment of health professionals emulated in every district that has a TVET campus.

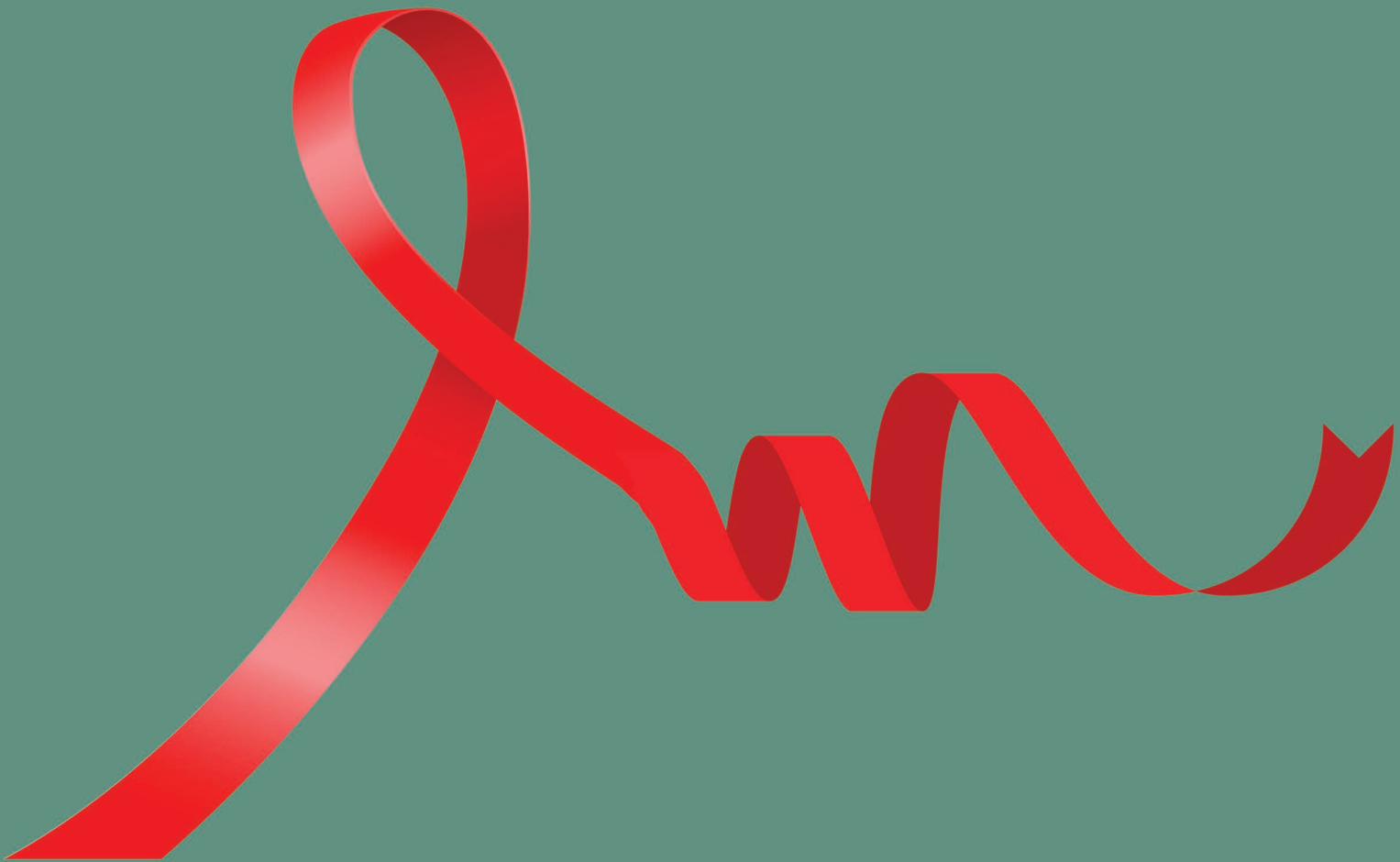
I would also like to thank staff members who have stepped forward to become peer education mentors and students who have volunteered as peer educators. The HEAIDS programme still has limited resources and would not exist without the generosity of thousands of individuals who are willing to contribute for very little personal reward.

The programme has done some vital regearing in 2016. We particularly appreciate the expansion of the team of provincial coordinators who are the first line of contact with HEAIDS for our colleges. I look forward to working closely with HEAIDS to achieve better results in 2017.



Hellen Ntlatleng
Deputy Chairperson
HEAIDS Advisory Board

1



About the Higher Education and Training HIV/AIDS Programme

Overview

1.1 Overview

The Higher Education and Training HIV/AIDS Programme (HEAIDS) assists public universities and technical and vocational education and training (TVET) colleges across South Africa to respond to the HIV and TB epidemics through their core activities of teaching and learning, research, innovation and knowledge sharing, and engagement with communities.

HEAIDS strives not only to reduce the impact of the HIV and TB epidemics on students and staff on campuses throughout the country but also to contribute to containing, reducing and ultimately overcoming these threats to the health of South Africans.

HEAIDS is an initiative of the Department of Higher Education and Training (DHET) and is undertaken by Universities South Africa (USAf, the representative body of public universities) and the South African College Principals Organisation (SACPO).

In 2016 it was primarily funded from the South African fiscus through an allocation that flowed through the DHET, grant funding from the National Skills Fund (NSF) and membership contributions from public universities and colleges. HEAIDS also benefitted from substantial funding from the Global Fund to Fight AIDS, TB and Malaria, and support from the German Development Agency, GIZ, and a private sector telecommunications company, Blue Label.

HEAIDS has been guided at a strategic level by successive national strategic frameworks on HIV, TB and sexually transmitted infections (STIs) and the objectives set out in these five-year plans.

Fiduciary responsibility for HEAIDS vests in USAf.

HEAIDS is accountable to the DHET, to all public universities (represented by USAf) and all public TVET colleges (represented by SACPO). These entities play a significant role in the Advisory Board, which guides and oversees the work of HEAIDS, and participate in the Executive Committee of the board. The board is chaired by Dr Max Price, Vice-Chancellor of the University of Cape Town. A full list of board members, who are drawn from a wide range of organisations, appears below.

1.2 Members of HEAIDS Advisory Board

Dr Max Price (Chairperson), University of Cape Town

Ms Hellen Ntlatleng (Deputy Chair), South African College Principals Organisation (SACPO)

Prof Quarraisha Abdool Karim, Centre for AIDS Programme of Research in SA (CAPRISA)

Dr Ramneek Ahluwalia, HEAIDS Programme Director, Universities South Africa (USAf), (Ex officio)

Ms Nomampondo Barnabas, International Union against Lung Disease and TB (IUATLD)

Dr Ahmed Bawa (Chief Executive Officer), Universities South Africa (USAf)

Ms Steve Letsike, South African National AIDS Council (SANAC)

Mr Mahlubi Mabizela, Department of Higher Education and Training (DHET)

Mr Khorombi Madzhie, South African College Principals Organisation (SACPO)

Ms Vuyokazi Mafilika, Department of Higher Education and Training (DHET)

Prof Thoko Mayekiso, University of Mpumalanga
Mr Avela Mjajubane, South African Union of Students (SAUS)
Mr Tshegofatso Moralo, National Education, Health and Allied Workers Union (NEHAWU)
Mr Sibhanana Ndlovu, South African College Principals Organisation (SACPO)
Dr Prins Nevhutalu, Cape Peninsula University of Technology
Dr Zukiswa Pinini, Department of Health
Ms Susan Preller, SA Business Coalition on HIV/AIDS (SABCOHA)
Mr Yonke Twani, South African FET Students Association (SAFETSA)

1.3 The National and Provincial Office

Senior Management

Dr Ramneek Ahluwalia, Director
Ms Managa Pillay, Senior Programme Manager: Curriculum Development
Ms Sinikiwe Sithole, Senior Programme Manager: Global Fund
Mr Macdonald Takundwa, Deputy Director: Corporate Services

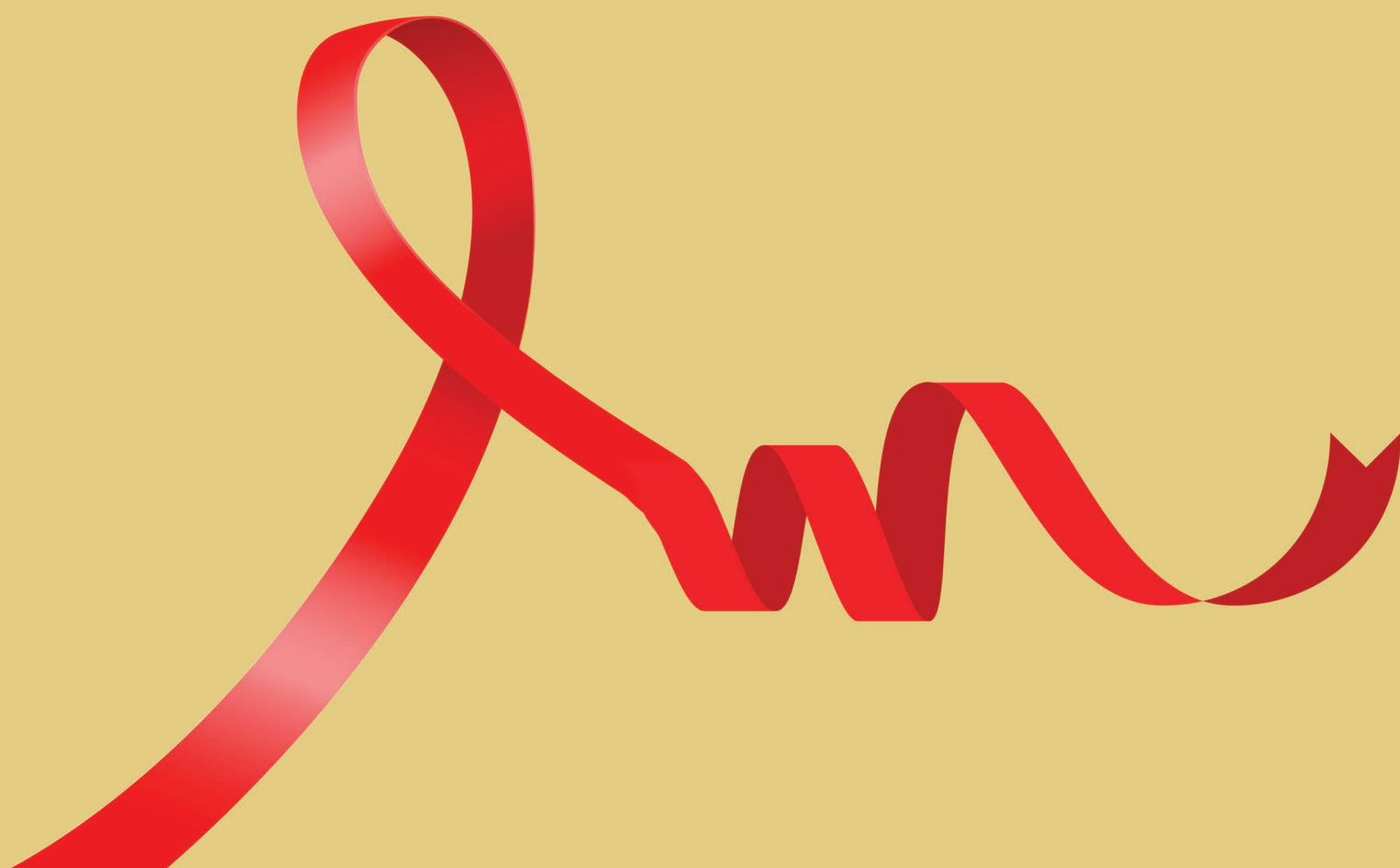
The National Team

Ms Miriam Behrendt, Technical Advisor: Media and Communications
Ms Theodora Chauke, Programme Administrator
Ms Lorinda Crouse, Executive PA to the Director
Ms Bonita du Plessis, Managing Editor: Future Beats
Mr Stembiso Magagula, Programme Coordinator
Mr Isaac Makore, Finance and Procurement Officer
Ms Zandile Mashaba, Communication and Events Officer
Mr Shibashiba Moabelo, Programme Coordinator
Ms Veronica Mqwathi, Programme Coordinator
Mr Alex Semba, Programme Manager: Universities
Ms Zamaswazi Shabangu, Programme Administrator
Ms Brenda White, Programme Manager: TVET Colleges

The Provincial Team

Ms Christina Dzimiri, Provincial Coordinator
Ms Gwynneth Makuwaza, Provincial Coordinator
Mr Oziel Mdletshe, Provincial Coordinator
Ms Xolelwa Ngqele, Provincial Coordinator
Ms Siphesihle Ngubo, Provincial Coordinator
Ms Thandi Ntuli, Provincial Coordinator
Ms Thokozani Nyawasha, Provincial Coordinator
Ms Marilize Olckers, Provincial Coordinator
Ms Gail Roman, Provincial Coordinator
Mr Pakiso Thulo, Provincial Coordinator
Mr Mawethu Zita, Provincial Coordinator

2



Overview by the director

Introduction



FTF Activation at Zwelitsha Campus students flanking Deputy Minister Mduduzi Manana, Dr Ramneek Ahluwalia, Dr. Mvuyo Tom, Vice Chancellor of the University of Fort Hare, Mr Luvuyo Ngubelanga, Lovedale College Principal

2.1 Introduction

The year 2016 proved to be a period of transformation for the Higher Education and Training HIV/AIDS Programme (HEAIDS). The organisation's energies were focused, to a large extent, on developing the organisational infrastructure necessary to carry the programme forward and allow it to flourish in the future.

In adopting this approach, HEAIDS aimed to address the long-term sustainability of the programme both in the university sector and the technical and vocational education and training (TVET) sector, and to fast-track the building of structures and capacity at TVET colleges which were only fully incorporated into HEAIDS in 2014.

Despite this strong developmental focus, the HEAIDS programme was able to sustain delivery of preventative health interventions to campuses countrywide in 2016 and achieve levels of HIV testing and TB screening comparable to those recorded in 2015.

- The number of First Things First (FTF) activations – which combine health promotion activities, HIV testing, and screening for various

other health conditions – rose to 713 in 2016. This was due to a strong demand for activations among TVET colleges. The massive expansion of activations from a mere 13 some five years ago has been a crucial success story within the HEAIDS programme.

- The total number of individuals who tested for HIV in 2016 or underwent screening for TB and other sexually transmitted infections (STIs) was slightly lower than the record number achieved in 2015. Nevertheless, the response remained substantial with 160 007 individuals testing for HIV and 155 234 screened for TB. FTF was affected both by the unstable climate on many campuses during the year and the amount of energy directed to capacity building and longer term sustainability of the programme.
- Peer educators on campuses are critical to the success of activations and to sustaining health promotion between these major events. In 2016 HEAIDS developed a new model for peer education training and coordination which addresses the challenge of sustainability by skilling and reimbursing peer education mentors among college staff and peer education coordinators at universities. The number of university and college peer educators trained in 2016 was 7 072. The growth of this aspect of the programme, from humble beginnings with a mere 337 peer educators in 2013, is also worth noting.
- Dedicated funding for curriculum development on HIV has paid off as this aspect of HEAIDS' work has gained traction in recent years. In the university sector about 850 academics and other staff members have participated in curriculum workshops since 2015. These workshops are run by interested academics for academics. They take account of the diversity of the student population and promote participative methods of teaching. In the TVET sector the

first major stream of curriculum development – life orientation – is now being implemented with the assistance of critical tools developed through HEAIDS to improve the HIV-related content of this curriculum.

- The Future Beats programme, which leverages community and campus radio stations to provide quality information on HIV and related matters and to combat stigma and attitudes that present barriers to prevention and treatment, continued its steady expansion. Since it started in 2014 Future Beats has provided training to 171 community and campus radio journalists. During 2016, the programme reached about 720 000 young listeners through 14 participating radio stations.
- An evaluation of the FTF programme, undertaken by the Human Sciences Research Council (HSRC), showed that key role-players at randomly selected colleges and universities rated the initiative extremely highly in terms of its relevance to their institutions, operational efficiency, output delivered relative to resources invested, and changes achieved on campuses. Researchers assigned an overall score of 86% to the qualitative assessment by stakeholders and 71% for a separate paper-based evaluation of the value for money represented by FTF.
- Further progress was made in 2016 in terms of HEAIDS' transition from short-term project-based funding to long-term funding and from international partners to domestic financing streams. The proportion of domestic funding rose to 68%. In addition to this, role-players in the public health service increased their contribution of human resources and healthcare consumables for HIV, TB and related care at universities and colleges.
- Towards the end of 2016 HEAIDS assumed the responsibility of facilitating a national initiative

to address the pervasive problems of sexual assault and gender-based violence on campuses across the country. At some institutions these matters had become a flashpoint in the volatile climate of student protest. HEAIDS' experience in opening up dialogue at colleges and universities on matters related to sexuality, sexual health and related social practices will serve the programme well in this new area of responsibility. HEAIDS considers that the complexity and significance of this matter warrant the establishment of a stand-alone programme to address it.

- Great progress was made in the TVET sector in establishing coordinating committees dedicated to the oversight and resourcing of HIV-related interventions. These institution-level HIV coordinating committees, are generally headed by the vice-chancellor or principal, and have played a critical role in advancing HEAIDS at universities. By the end of 2016 these structures were functioning at 90% of TVET colleges and we are confident that their leadership will make an enormous difference in years to come.

2.2 Contributing to the national HIV and TB response

At a strategic level, HEAIDS has always been strongly aligned with successive national strategic plans on HIV, TB and STIs and motivated by the ambitious national targets for prevention, treatment and a human rights-based approach to the epidemics.

The national approach is, in turn, strongly influenced by global strategies and best practice advocated by the Joint United Nations Programme on HIV/AIDS (UNAIDS) and the World Health Organization (WHO).

Since FTF is HEAIDS' flagship programme and is built around mass voluntary HIV counselling and testing, HEAIDS has found enormous relevance in the UNAIDS 90-90-90 fast-track strategy. This set the global goals of 90% of populations knowing their HIV status by 2020, 90% of those living with HIV receiving antiretroviral treatment (ART), and 90% of those on treatment attaining viral suppression.

In 2016, however, on the eve of the International AIDS Conference, held in Durban in July 2016, UNAIDS released a sobering report on the prevention gap. This highlighted the fact that earlier progress in reducing new HIV infections and AIDS-related mortality had stalled. Not only was the 90-90-90 model not yet realising the "prevention dividend"

it promised but it was clear that other prevention strategies must be pursued more vigorously.

This warning certainly struck home in South Africa where we were aware that about 270 000 individuals – mostly young people – would become infected during the course of 2016 despite our country having the biggest ARV treatment programme in the world and probably the world's largest no-charge condom supply system.

The national campaign for young women and girls, *She Conquers*, was launched by Deputy President Cyril Ramaphosa in June 2016. This takes a holistic approach to HIV prevention among young women, addressing

matters of discrimination, gender-based violence and the key factor reproductive choice. The critical need for such an initiative was underscored by the entry of the term "blessers" into our everyday vocabulary and the sight of young female students facing arrest for protesting rather than remaining silent about sexual violence on campus. The synergy between this campaign and the HEAIDS programme is self-evident and we will strongly support it. All relevant information, education and communication materials of HEAIDS carry the logo of the *She Conquers* campaign.

HIV testing results obtained during activations show considerable variation from campus to campus in terms of the proportion of young people who test HIV positive. At some institutions the number is disturbingly high and we are constantly reminded of the HIV threat to our country's youth, to entire families who invest in their most promising children as security for their joint future, and to the skills base of this country.

More than three decades into the HIV epidemic, an HIV programme geared to the unique needs of the higher education system is clearly not merely a "nice to have". It remains critical to the health of upcoming generations, is deserving of support, and it must innovate to meet the needs of young people in a rapidly changing world.

2.3 Overview of the sector

About 1.7 million students are enrolled at public universities and public TVET colleges in South Africa. Although there are fewer universities than colleges, the former tend to be very large institutions and account for about 58% of the total enrolment. The colleges comprising the TVET sector range from small institutions of fewer than 1 500 students to large institutions with several thousand students spread over two or three campuses.

Table 1: Institutions, staff and students in public university and public TVET sectors

	University sector	TVET college sector
Number of institutions	26	50
Number of staff members	51 310	16 229
Number of students	985 212	709 535

Source: Unpublished statistics for 2015 supplied by DHET.

While universities generally have the critical mass of students necessary to develop viable, full-time health services on campus, this is seldom the case for TVET colleges. A key feature of our work with TVET colleges is enhancing partnerships with local health service providers so that their students and staff have access to clinical care when needed.

2.4 The HEAIDS model of practice

Drawing on research and its years of experience in the sector, HEAIDS has forged a model of practice designed to meet HIV prevention, care and treatment needs as well as some broader primary health needs of various sections of the student population.

The centre of the model is the FTF programme, which focuses on prevention and early detection of HIV, TB and a range of health conditions and health risks. The underlying premise of this programme is that it is the first priority of every South African to look after his or her health.

Through FTF HEAIDS aims to build a culture where youth take an interest in their health, become knowledgeable about it and incorporate testing and screening into the normal course of their lives.

HEAIDS enables students to assume responsibility for their health by conducting FTF activations at all campuses. The goal is to undertake at least two activations a year on every campus and this target is within reach. Activations differ slightly from campus to campus but each one encompasses:

- Health promotion, including condom promotion.
- Testing and screening for various health risks and health conditions.
- Individual counselling of students in relation to health matters.
- Referral for treatment and care, further testing and medical male circumcision.

Initially FTF focused exclusively on HIV, TB and STIs but it has begun to include contraceptive services and screening for non-communicable diseases (such as diabetes) and certain forms of cancer.

When an FTF activation is requested by a university or college, this institution becomes the primary implementation partner. However, many other partners in government and civil society are necessary in order to offer a full package of interventions during the activation.

Figure 1: The HEAIDS model of practice



In addition to FTF, HEAIDS facilitates four focused programmes that are run throughout the year. These programmes are advanced mainly through activities organised by a corps of peer educators that has been established on campuses. The content of each programme is suggested by its title:

- Women's Health Empowerment.
- Men's Health.
- Drug and Alcohol Abuse Prevention Programme.
- LGBTI Programme.

Peer educators are primarily responsible for mobilising participation in FTF, and convening discussion groups and dialogues related to the programmes listed above. Their efforts are reinforced through two additional delivery platforms:

- The Future Beats Youth Development Programme which uses radio and social media to engage students on relevant topics.
- The Curriculum Development Programme which strives to build student competencies on HIV (for later application in the workplace) through the curricula taught at universities and colleges.

In reality, FTF and the four additional programmes overlap and reinforce each other. Activations may include a specific emphasis on one of the other programmes if this is especially relevant to the campus where the activation takes place.

2.5 The HEAIDS model at work

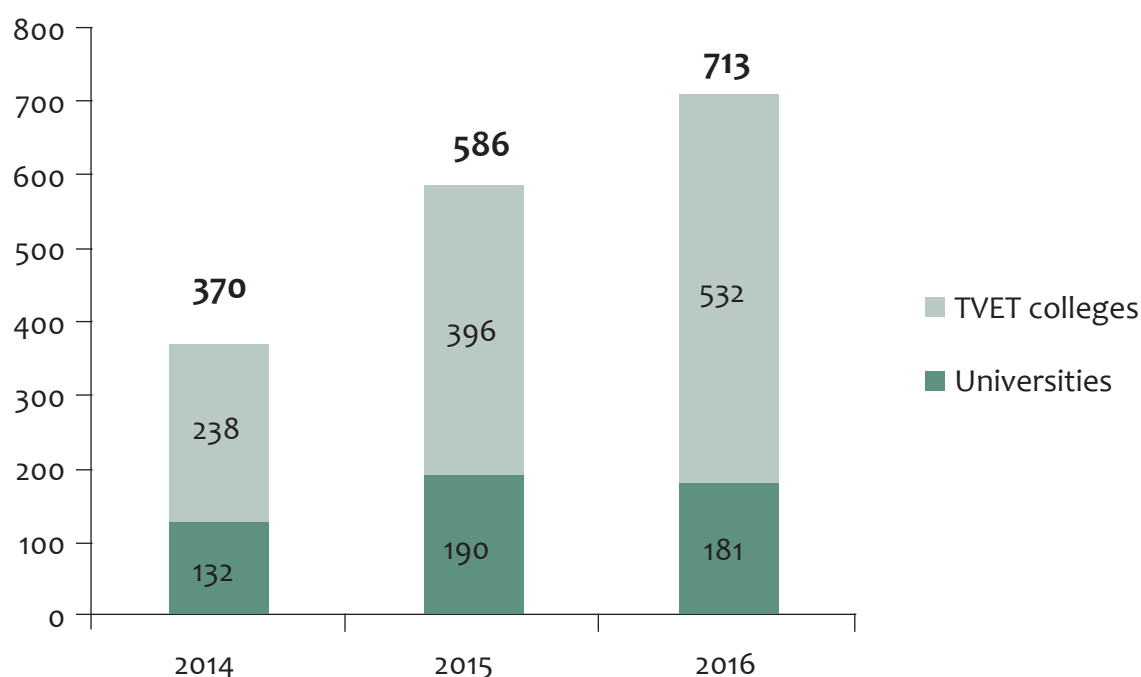
2.5.1 Campus activations

FTF activations are initiated by individual universities and colleges and supported by HEAIDS in various ways. The total number of activations and the proportion of campuses that host repeat activations is essentially determined by the demand at institution level.

In 2015, the FTF model showed a huge improvement in performance, both in terms of the number of activations undertaken and the number of students and staff members who responded by finding out their HIV status. In 2016, the combined figures for universities and colleges showed a sustained upward trend in the number of activations but a slight drop-off in testing numbers.

While the programme remained hugely beneficial to many thousands of individuals, our inability to sustain growth in HIV testing was of some concern. When we look more closely at the figures, we note different trends in the university and TVET college sectors. (Figures 2 and 3)

Figure 2: Number of activations at universities and TVET colleges 2014 - 2016

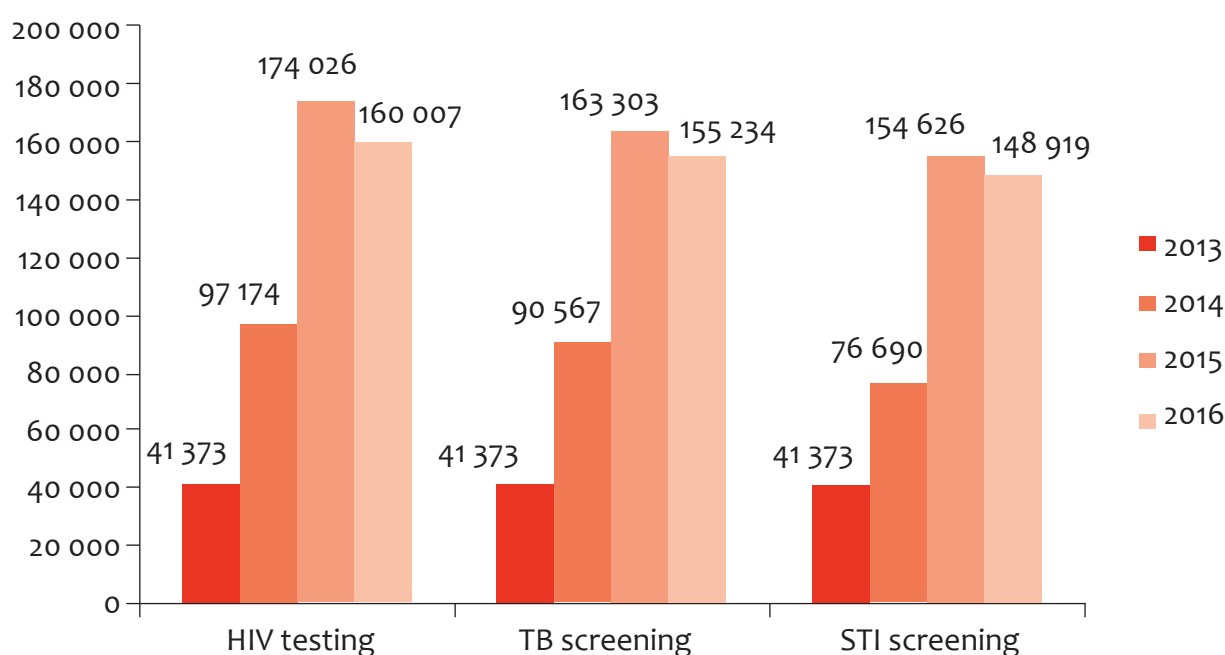


In the university sector, there was a slight drop in the number of activations and a proportionate drop in the number of individuals who underwent testing and screening. In contrast, at TVET colleges the number of activations increased substantially and yet the number of people who responded by taking an HIV test and getting screened for TB and STIs dropped.

The reasons underlying these distinctive patterns of output and uptake were quite different.

Programme performance at universities was affected largely by factors external to the HEAIDS programme. These included the student protests which affected many universities and were spread over quite an extended period. The resultant closure of campuses and climate of uncertainty were not conducive to health promotion activities and some activations were inevitably postponed or cancelled. Changes in international funding priorities for HIV also affected the participation of some civil society partners who had played a key role in FTF in previous years. Global Fund grants to universities, which had been so enabling in 2015, came to an end during 2016 and this constrained activities.

Figure 3: Numbers tested/screened at universities and colleges 2013 - 2016

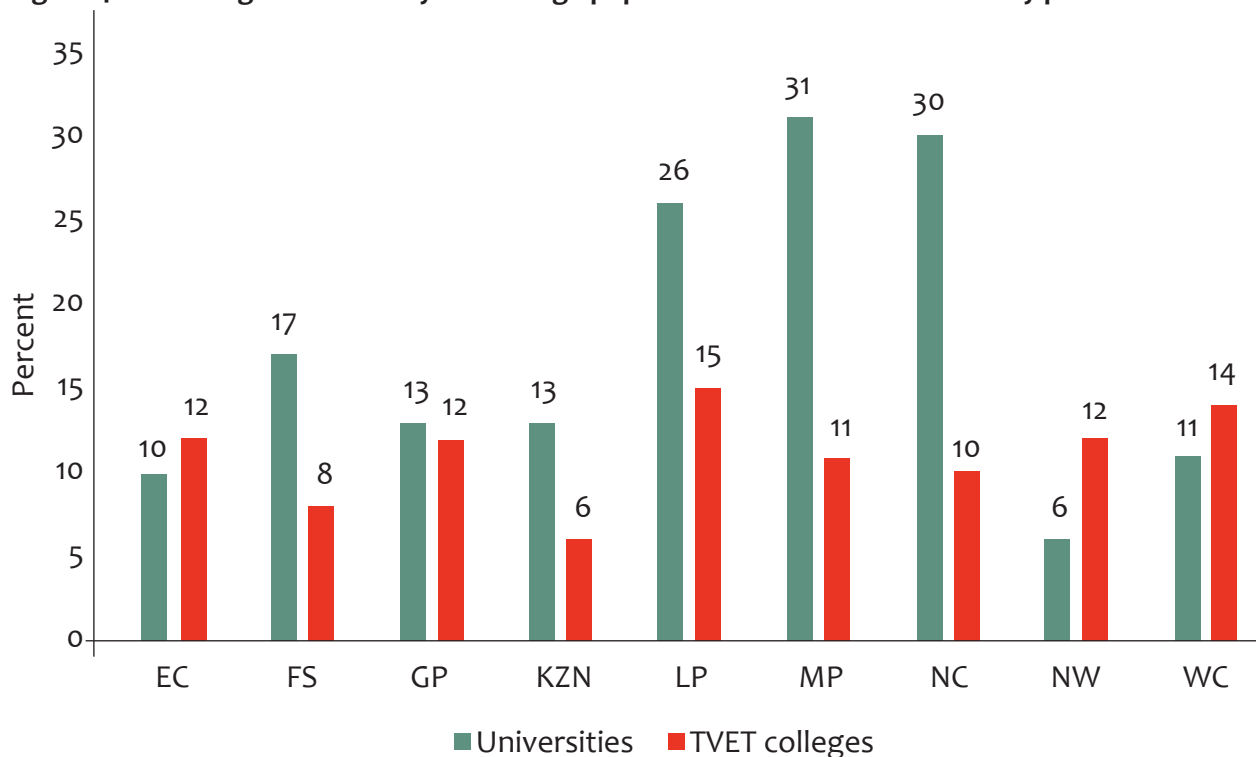


In the TVET sector it is clear that the enthusiasm and opportunity for expanding the reach of testing services existed, so the missed opportunity for expansion requires further examination. Part of the story is that HEAIDS is now reaching smaller campuses and the uptake of testing at individual activations is understandably lower there. However, there are a number of factors internal to the programme that might also have had an impact:

- The demand on HEAIDS to provide support for activations outstripped the capacity of our provincial coordinators and in KwaZulu-Natal the position of provincial coordinator was vacant for a few critical months. These constraints were recognised early in the year and rectified by July, however the full benefit of expanding the coordinator team will only be felt in 2017.
- Organisational capacity within TVET colleges was also limited and, once again, the identification and training of peer educator mentors at each college and the development of HICCs really only began to bear fruit late in the year.
- Necessary though this structural development and capacity building was, it diverted resources within HEAIDS and on campuses and temporarily compounded the problem of delivery to students.
- Finally, the late delivery of promotional items for FTF activations also impacted on the overall appeal of these events and may have resulted in lower student participation.

Figure 4 indicates the percentage of the total university and college population in individual provinces that was tested during FTF activations or at campus health facilities. Coverage is highest where the challenges are relatively simple – for example, in the university sector in provinces with a small number of relatively small institutions.

Figure 4: Percentage of university and college population tested for HIV in 2016 by province





Peer educator distributing condoms at FTF activation

2.5.2 Condom distribution

The use of condoms remains the mainstay of HIV prevention among HEAIDS' primary beneficiaries. Many students are not yet in stable relationships and research suggests that condoms are most frequently used in new relationships and/or casual sexual encounters. Convenient access and affordability are also important determinants of condom use.

During 2016, campus HIV programmes, health clinics and peer educator networks distributed nearly 15.8 million government-issue male condoms on college and university campuses. In addition, more than 106 000 female condoms and about 70 000 tubes of lubricant were distributed.

The new range of coloured and scented government-supplied condoms, marketed under the MAX brand, became available during 2016. The previous year the higher education sector had played a key role in a pilot project to test these coloured condoms and establish whether they were more acceptable to users than the old government-issue condoms.

The coloured and scented condoms were enthusiastically received by students during the pilot stage and their popularity carried through into 2016. There were, however, constraints on the supply

of condoms in this reporting year. During the pilot testing of the new condom, the supply of condoms to higher education institutions was ring-fenced. In 2016, however, universities and colleges sourced condoms from the stocks available through provincial health departments, sharing the available stock with other important outlets, such as clinics and NGOs.

2.5.3 Peer education training

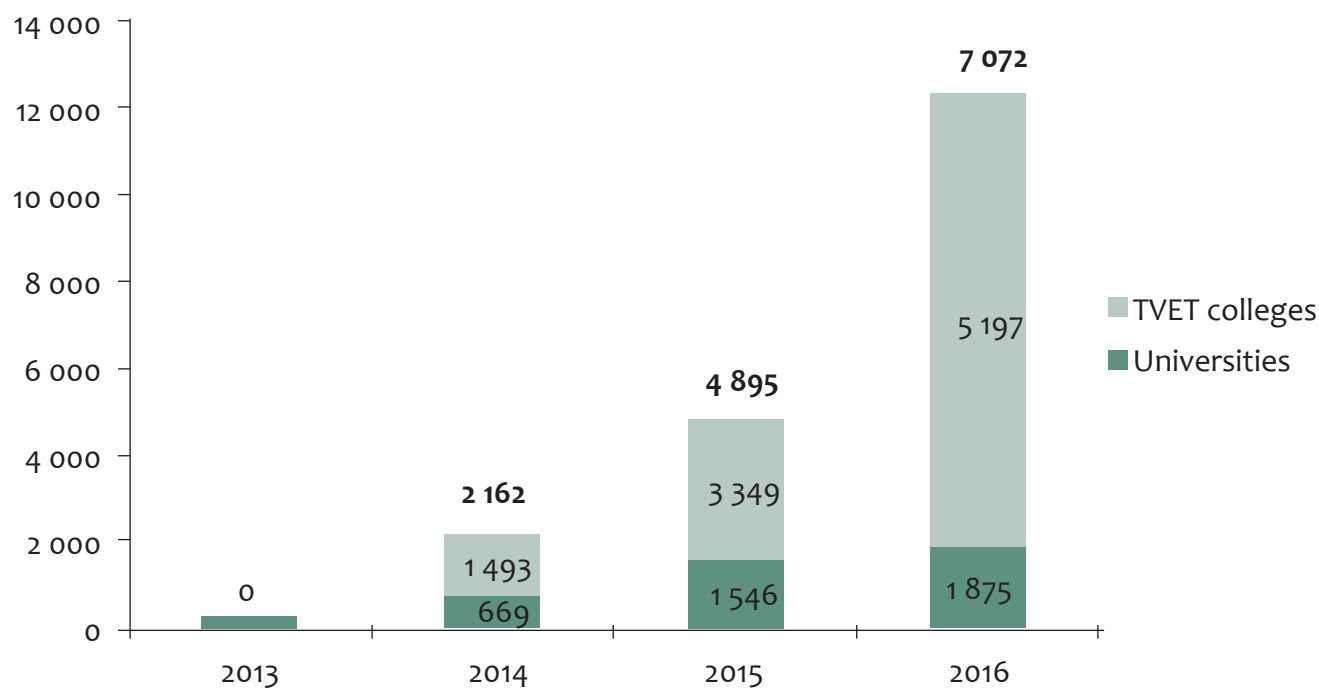
In 2016 HEAIDS significantly increased its investment of time, energy and resources in the peer education system that underpins many aspects of its work. This was not simply a matter of scaling up training courses for peer educators, but fundamentally rethinking the model for producing peer educators, coordinating their activities and providing them with support for optimal performance.

While transforming the peer education system, HEAIDS used a combination of existing training opportunities to ensure a strong flow of new peer educators into HIV programmes at all universities and TVET colleges. In 2016 HEAIDS trained a record-breaking number of peer educators and mentors: 12 342.

A comprehensive peer education training manual has been developed to ensure standardisation of training. The manual includes content that enables peer educators to support FTF activations, with their focus on HIV testing and health screening, as well as all other programmes that comprise the HEAIDS model: the Men's Health Programme, Women's Health Empowerment Programme, LGBTI Programme, and Alcohol and Drug Abuse Prevention Programme.

It deals not only with factual knowledge, but creates sensitivity to the beliefs, attitudes and social norms that stand in the way of healthy living. A section of the manual is devoted to the mentoring of peer educators.

Figure 5: Number of peer educators trained at universities and colleges 2013 - 2016



HEAIDS prides itself on the fact that the “foot soldiers” of the programme are young, committed volunteers. At most universities, established HIV programmes ensure a productive level of coordination of peer education activities. However, at other universities and most TVET colleges there are no structures to provide direction and support and therefore peer education energies have tended to wane.

The other challenge that faces the programme is the transience of the student population. The average peer educator in this sector is likely to offer two – or at most, three – years of service before graduating. This means that the system has an unusually high need for renewal and that the cycle of producing trained volunteers never really ceases. Stable structures are needed to ensure continuity and preserve institutional memory.

HEAIDS has devised a new model that will address these needs by training and remunerating peer education mentors at every college in the TVET sector and peer education coordinators at universities.

For example, the TVET model will ensure that there are two trained peer education mentors on every college campus. These mentors will be selected from academic staff who teach relevant subjects, such as life orientation. They will be trained and provided with the tools to train peer educators. They will receive additional remuneration as this work extends above and beyond their normal duties. Mentors will significantly strengthen institution-level implementation of the HEAIDS programme which is mainly in the hands of student support service managers and student liaison officers.

It is feasible for this peer education training model to be taken to scale, with HEAIDS provincial coordinators training 400 mentors at TVET colleges, each mentor training 10 peer educators a year and the corps of 4 000 peer educators produced through this system reaching 400 000 students during the course of the year.

The pilot phase of this new TVET model commenced in Gauteng and KwaZulu-Natal in 2016 with dedicated funding from the Global Fund. HEAIDS will monitor its strengths and weaknesses and refine the approach as it is introduced in the remaining provinces.

The university model, which will be implemented somewhat later, targets the annual training of 5 000 peer educators in order to create capacity to reach out to 500 000 students and staff members each year.

2.5.4 Future Beats Youth Development Programme



Future beats in-house training at Durban University of Technology, Radio DUT

The Future Beats programme utilises community and campus radio stations not only to build knowledge of HIV, TB and other health conditions but also to explore the social and cultural issues that impact profoundly on health and wellbeing.

Radio presenters and journalists are at the heart of this initiative and the programme enhances their journalistic skills so that they have the tools to deal with sensitive topics. It also deepens their understanding of complex matters, such as HIV-related stigma and sexual violence.

Future Beats was launched in 2014 and each year it has increased the number of radio stations in the programme. In 2016, the programme encompassed 14 community and campus radio stations with an estimated listenership of 720 000. During the year, 84 journalists attended various training workshops, bringing to 171 the total number of journalists benefitting since the programme's inception.

Through skilled and sensitive radio presenters and writers, standard health strategies can be transformed into locally relevant discussions, using languages that people feel confident using.

A formal evaluation of the programme, in terms of its impact on listeners, was conducted during the year and its findings are expected to be reported at the South African AIDS Conference in July 2017. Preliminary results suggest radio programmes linked to Future Beats were associated with stimulating discussion among friends about sexual subjects that are usually avoided and increasing uptake of services at campus and community health facilities.

2.5.6 Driving a national strategy on sexual and gender-based violence

Higher education institutions have grappled with how to respond effectively to sexual and gender-based violence on and around their campuses. The desperation and anger of students about this disturbing aspect of campus life exploded into protest at a number of institutions in 2016. HEAIDS, too, has been deeply concerned about sexual and gender-based violence which are widely acknowledged to be fuelling the extremely high HIV infection rate among young women.



Prof Ahmed Bawa, Ms. Navchaa Suren, Ms Jackie Dugard, Dr Ramneek Ahluwalia, Honourable Mduduzi Manana, Prof Rachel Jewkes and Ms Managa Pillay

In October 2016, at the request of Deputy Minister Manana, HEAIDS convened a national stakeholder dialogue on the subject. The intention was to launch a process that would result in a strategy to tackle sexual and gender-based violence in the higher education sector with greater resolve and effect.

The outcome was the creation of the Higher Education Sexual and Gender-based Violence Technical Task Team charged with:

- Developing a sector-wide policy framework on addressing sexual and gender-based violence.
- Overseeing training and development of a wide range of students and staff representatives who are critically placed to assist in implementation of the policy framework.
- Undertaking consultation, advocacy and dialogue in order to build consensus and ensure that the needs of students and staff at risk are met.
- Enabling changes to campus-based health and counselling services to ensure they are better equipped to undertake preventive activities and

- provide accessible post-assault services.
- Developing a monitoring and evaluation system to measure the impact of the sectoral strategy.

The task team is jointly chaired by Prof Rachel Jewkes of the South African Medical Research Council, who is responsible for the policy framework sub-committee, and Ms Sianne Abrahams of the University of Cape Town, who heads the sub-committee on strategic programmes. Other members of the task team are: Mr Nick Balkrishen of Gert Sibande TVET College, Prof Chrissie Boughy of Rhodes University, Ms Dorcas Gcabashe of Durban University of Technology, Ms Noluthando Hlubi of SAFETSA, Ms Sesi Mahlobogoane of DHET, Ms Nazeem Mahomed of the DHET, Ms Eyethu Makeke of SAUS, Ms Tsholo Moloi of the Department of Social Development, Ms Nombulelo Msikinya of the Department of Women and Child Development, Ms Andile Mthombeni of Wits University, Ms Caroline Nyamayemombe of UN Women, Ms Mary Peters of Elangeni TVET College, Ms Navchaa Suren of UNFPA, and Prof Shirley Walters of DHET.

HEAIDS has assumed the role of coordinating and supporting the structure and will offer extended support to universities and colleges in the implementation of the strategy that is developed. HEAIDS Technical Advisor for Special Projects, Ms Managa Pillay, will lead the process.

Ms Lisa Vetten, nationally respected for her sustained work in the area of gender-based violence, has been appointed technical assistant to the task team and will be responsible for the first draft of the framework document. UNFPA has provided funding for specialist technical assistance, while UN Women has also provided significant support for this initiative.

Together with the Deputy Minister, HEAIDS has planned a series of campus dialogues on sexual and gender-based violence beginning soon after the start of the 2017 academic year. The intention is to signal the programme's seriousness about tackling this threat to the wellbeing of students and staff, to listen and learn from campus communities, and pave the way for the introduction of the strategy.



TVET Colleges Capacity Development for
Life Orientation lectures training

2.5.7 Curriculum Development

In 2015 HEAIDS made substantial progress in the university sector in terms of providing support and building the skills of academic staff for the integration of HIV-related content into a range of curricula. With grants from the NSF in place for individual universities

and early adopters of HIV curriculum integration sharing their methods with their peers, the university programme made steady progress in 2016.

There was also forward movement in the TVET sector, utilising the findings of the baseline study that had been undertaken the previous year. This study assisted in identifying three work streams for the development of HIV content: mining and engineering, agriculture, and life orientation.

Since life orientation already deals with sexuality, health and HIV, the approach is to strengthen and standardise the content. To achieve this HEAIDS developed an assessment guideline and a workbook which will assist colleges to enrich their teaching of HIV-related content in the life orientation curriculum. The workbook was piloted in three provinces before being adopted nationally. In early 2017 HEAIDS will offer capacity building in the use of the assessment guideline for level 4 life orientation lecturers.

An intensive series of meetings were held with management and relevant academic staff at colleges in order to pave the way for the introduction of the two other curricula.

The three-year grants made by the NSF for HIV curriculum development have been critical to the advances made. They have enabled academic and teaching staff to interact and share insights, bringing enthusiasm and movement to an area of work that had been stagnant.

2.6 Research and knowledge

Over the years, HEAIDS has followed an evidence-informed approach to programme development, undertaking its own research where necessary, and has instituted formal evaluation of aspects of its work. The organisation has also participated regularly in national and international conferences with a view to sharing its knowledge.

In 2016, our major research studies were both programme evaluations:

- The evaluation of FTF was undertaken in 2015 by the HSRC and the findings were published in 2016. Its purpose was to establish whether FTF was relevant, efficient in terms of delivery, and represented good value in terms of output relative to investment. The researchers assessed value for money partly by analysing HEAIDS performance and financial reports on FTF. They concluded that FTF represented value in the sector and was regarded as extremely relevant and efficient.
- The Future Beats programme was evaluated in 2016 in terms of its impact on listeners of participating campus and community radio stations. The results will be published in 2017. An earlier evaluation which documented the impact on radio stations was very positive.



Universities Programmes Manager,
Alex Semba at the Future Beats inhouse training
Durban University of Technology

HEAIDS was fortunate to be selected to make three poster presentations at the International AIDS Conference, the world's premier gathering of researchers, policy makers, programme planners, advocates and service providers in the HIV and TB fields. These presentations dealt with:

- The utilisation of the Global Fund grant system

to deliver HIV programmes in the higher education sector in South Africa.

- The HEAIDS experience in capacity development of academics as a way to address inter-sectoral aspects of HIV.
- HIV prevention programmes at TVET colleges, drawing on the knowledge, attitudes and behaviour study undertaken by the HSRC.

A gratifying aspect of these presentations was the interest shown by delegates from other African countries and their desire to institute something similar.

HEAIDS also hosted a satellite session at AIDS 2016 which centred on the evolving HEAIDS practice model. About 500 people came to hear the panel of speakers which comprised: Deputy Minister Manana, HEAIDS Director Dr Ahluwalia, Professor Geoffrey Setswe of the HSRC, epidemiologist Professor Quarraisha Abdool Karim, who serves on the HEAIDS Advisory Board, co-chair of the South African National AIDS Council (SANAC) and Steve Letsike also on HEAIDS Advisory Board.

The HEAIDS network of national and provincial forums and communities of practice also presented valuable opportunities for knowledge sharing. Knowledge dissemination is included in as many meetings as possible because a common foundation of knowledge helps to build the coherence of the programme.

2.7 Communication and stakeholder engagement

HEAIDS uses a variety of platforms to interact with stakeholders and inform the broad public about its work.

HEAIDS considers its work is of public interest and strives to ensure that journalists in the mainstream media are aware of the programme and carry stories on its activities from time to time. HEAIDS' experience in working with young people over the years, and the range of research we have carried out on HIV and related matters, has been the basis of participation in various media debates.

In 2016 the editorial coverage on HEAIDS in the mass media was valued at R7.36 million using values equivalent to advertising rates. We featured in a total of 151 print and online articles and broadcast news items. About half the media coverage we received was in the broadcast media. This requires a higher investment of time and energy, as a media release seldom suffices and individual interviews are required. However, we believe the effort is worth it in order to reach a more diverse audience.

Stakeholder engagement occurs at many levels in HEAIDS and its programme managers and provincial coordinators play an invaluable role in forging partnerships and keeping important networks and individuals informed about developments in the organisation.

In addition to this HEAIDS organises special events – often in partnership with universities and colleges, participates in major national and inter-sectoral events, and produces a range of publications for distribution to our stakeholders. In 2016 these stakeholder relations activities included:

- Contributing to activities in the AIDS 2016 Global Village, which was open to the public as well as conference delegates. HEAIDS organised interactive sessions for young people at the South African government's stand in the village and formed part of the SANAC exhibition. Several universities created quilts for the national quilt exhibition at the conference.
- The organisation of five provincial FTF activations in partnership with selected TVET colleges. The presence of Deputy Minister Manana at most of these, together with leading provincial representatives, spoke volumes about the value government attaches to the effort being made by colleges to strengthen the national HIV programme.

2.8 Technical support to institutions

HEAIDS is ultimately a programme that is implemented by universities and colleges, with the help of partners and service providers. The FTF activations, training sessions for peer educators, curriculum workshops, distribution of condoms, and HIV-related radio programmes simply would not happen without the support of university and college management, which is expressed through institution-based HIV coordinating committees, and passionate and skilled operational leadership at campus level.

It is equally true, however, that HEAIDS plays a significant facilitating role through its programme managers and provincial coordinators. This takes many forms, from lobbying for leadership support and additional resources, to strategic guidance, match-making between institutions and potential service partners, strengthening the knowledge and skills of campus role-players, and providing collateral for campaigns.

Programme targets student HIV infections: 1 000 a day

Pretoria News 31 Jan 2017 GOITSEMANG THABYE goitsemang.thabye@inl.co.za

WITH 380 000 new HIV infections a year – 1 000 a day – and where 2 000 young women get infected each week, getting the message through to South Africa's youth is more crucial than ever.

This, as the Higher Education and Training HIV/Aids (HEAIDS) Programme kicks off its seventh annual campus-based HIV, STI, TB and related health and wellness initiative.

HEAIDS was established in 2000 as a partnership between the then Department of Education, the South African Universities Vice-Chancellors Association and the Committee of Technikon Principals.

Since then, it has taken shape in three phases. Phase one ran from 2001 to 2005, during which it conducted pilot studies, gathered data and had impact analyses. It also established decision-making and programme-management structures, created loose networks and accessed re-

sources.

Phase two, from 2005 to 2009, established the need to channel the response through higher education institutions by focusing on teaching and learning, research and innovation, and community outreach.

And in the last phase, initiated in 2012, emphasis was on the implementation of comprehensive combination prevention in line with the National Strategic Plan 2012-2016.

Currently, with the national HIV infection rate peaking at the age of 25, the programme is seeking to reach the youth transitioning from school to the higher education environment.

HEAIDS director Dr Ramneek Ahluwalia noted that the change in environment often saw youngsters meeting new people and adopting new approaches to personal and sexual relationships at a delicate age.

The initiative spans 400 campuses of 76 universities and colleges

in the public higher education sector.

"The excitement of exploring novel territories is not risk-free, though, as HIV prevalence among South African women is nearly twice as high as men of the same age," said Ahluwalia.

"This means that they are vulnerable to acquiring HIV at this pivotal moment in their lives. The national HIV infection rate peaks at the age of 25, hence the focus of many HIV prevention efforts are people of tertiary education age."

The health of students is as much a factor in the outcome of studies as is access to tertiary education and uninterrupted studying, said Ahluwalia.

"Most youth understand that HIV, STIs and TB are preventable, yet these conditions hit thousands of students and stifle progress and prospects, possibly for life.

"We empower each student to take ownership of their health and

future-proof their educational and professional success."

In 2015, through the use of activations, 175 000 students and young people tested for HIV and screened for STIs, TB and other health conditions. A record 25 million coloured, scented condoms were distributed to students and young people.

Research conducted by the Human Sciences Research Council in 2014 and 2015 credited the HEAIDS programmes – particularly the First Things First programme – with being a valuable source of information and awareness-building, that supports health-seeking behaviour, and encourages testing, collaborations and resource-allocation, the director said.

The First Things First programme was recently awarded the Top Health Programme in Africa, an accolade HEAIDS prides itself on. It meant they were one of the most far-reaching and impactful national health programmes.

Other programmes are the Men's Health and Empowerment Programme, and the Women's Health and Empowerment, Alcohol and Drug Abuse Prevention and Academic Capacity Development programmes.

There are also men who have sex with men and lesbian, gay, bisexual, transgender and intersexed people's (LGBTI) programmes, the Future Beats Youth Development programme and HIV prevention through campus radio and social media.



Pretoria News Article, 31 January 2017

Another important role of HEAIDS is organising forums where universities and colleges can learn from each other by sharing their experiences. These forums also provide HEAIDS programme managers with critical information about the realities and priorities at campus level and help to keep planning attuned with the needs of institutions.

All HEAIDS' national and provincial staff are constantly on the move, meeting with institutional management and HIV programme implementers, convening provincial meetings, forging relationships with provincial HIV councils, and liaising with departments of health at provincial and district level.

The support requirements of the TVET sector are greater than those of the university sector. Not only are there more TVET campuses spread widely across the country, but many colleges are too small to have the specialised personnel that most universities are able to leverage for their HIV and health interventions.

During 2016, the provincial coordinators paid 532 visits to colleges. Although this was 8% fewer than during the previous year it still represented an enormous workload.

In mid-2016 HEAIDS expanded its team of provincial coordinators from eight to 13 in order to keep pace with the demand for support and will be much better set up to answer colleges' needs in 2017. The duties of provincial educators going forward will include the training of and support of peer education mentors.

2.8.1 Grants to universities and TVET colleges

Because the HEAIDS programme rests on a partnership between the programme office and individual universities and colleges, several major grants allocated to HEAIDS are for further disbursement to individual universities to enable them to participate more fully in the programme.

2016 saw the conclusion of a valuable three-year grant from the Global Fund which was managed by Networking AIDS Community of Southern Africa (NACOSA). This played a very significant role providing universities with resources to implement the various programmes that fall under the HEAIDS umbrella. Among other things, this grant also funded the important survey on knowledge, attitudes and behaviour of LGBTI students and staff in 2014.

The Department of Health has helped to fill the funding gap that opened up with the ending of the Global Fund-NACOSA grant by making an amount of R1.85 million available for allocation to universities to sustain their activities to address HIV, TB and STIs. During the course of 2016, universities were invited to submit project proposals and allocations ranging in value from R50 000 to R100 000 were made to 21 universities. The first amounts will be paid over to universities early in 2017.

Currently, HEAIDS has the benefit of another Global Fund allocation, managed through the AIDS Foundation of Southern Africa (AFSA), which also plays a developmental role at institutional level. This grant has been particularly valuable in facilitating the pilot implementation of the new peer education training model at TVET colleges in Gauteng and KwaZulu-Natal.

The grants from the National Skills Fund (NSF) which have invigorated HIV curriculum development at universities and colleges, will run through 2017. In the case of universities, individual institutions receive allocations and this access to resources has given academics who are committed to HIV curriculum development the opportunity to share their skills and insights.

2.9 Summary of financial status

The total income for the HEAIDS programme in 2016 amounted to R42.4 million – an increase of 20% on the previous year – and spending against this budget totalled R30.4 million. It should be noted that some grants were made for the period April 2016 to March 2017, and therefore still had three months to run at the time this report was prepared.

One of HEAIDS' goals has been to increase domestic sources of funding and to focus on sustainability of funding. In 2016, no less than 68% of our total income was derived from domestic public sector funding. The main sources of income in 2016 were:

- NSF grant for curriculum development (21%).
- NSF grant for support to the TVET sector (20%).
- The Global Fund grant channelled through NACOSA (15%).
- The Global Fund grant channelled through the AFSA (14%).
- Membership fees from universities and TVET colleges (8%).
- GIZ grant for Future Beats (2%).
- Blue Label Telecommunications (2%).

2.10 Acknowledgments and appreciation

HEAIDS remains vibrant and relevant only because so many committed people have embraced this initiative. Decentralisation and partnership are the essence of our working style and this means none of us experiences the programme in its totality. It also means it is impossible for me to thank individually many of our most valuable supporters. Please be assured that every contribution is greatly appreciated. We would especially like to recognise those who are devoted full-time to building the HIV response in universities and colleges.

At national and international level I extend appreciation to:

Minister of Higher Education and Training Dr Blade Nzimande	For his leadership in the field of higher education and continued support of HEAIDS.
Deputy Minister of Higher Education and Training Mduduzi Manana	For continuing to champion the HEAIDS programme and effectively becoming its public face.
Minister of Health Dr Aaron Motsoaledi	For acting consistently in the spirit of inter-sectoral collaboration and unlocking precious health resources for students at institutions.
The Global Fund, the National Skills Fund, the GIZ, the Department of Health and Blue Label Telecoms	For the funding that empowers HEAIDS and institutions to undertake this work and for their interest in the programme's progress.
Members of HEAIDS Advisory Board	For sharing their wisdom and experience generously with HEAIDS and continuing to prioritise the programme in the face of other pressing demands.
Universities South Africa and the South African College Principals Organisation	For consistent support, facilitating access to their members and, in the case of USAf, providing fiduciary oversight of the programme.
Senior officials in relevant sections of the Department of Higher Education and Training and the Department of Health	For their expertise and practical assistance in various areas of programme activity.
National NGOs in the field of HIV prevention and care	For working by HEAIDS' side in a spirit of camaraderie and with a shared vision.

At provincial and district level, HEAIDS' appreciation goes to:

MECs for health and mayors	For visible support and for facilitating the flow of key resources for campus healthcare.
Members of provincial workings groups	For their commitment to strengthening the programmes, mobilising resources and sharing insights on practice.
Provincial and district health officials and their staff	For building campus health and HIV interventions through resourcing and hands-on service provision.
Provincial AIDS councils	For admitting HEAIDS as a member and facilitating partnerships that have strengthened the programme.
NGOs working in the province or district	For providing essential support during FTF activations, and assistance in building capacity on campus.

At university and college level I would especially like to acknowledge:

All vice-chancellors and TVET college principals and other members of institutional management, especially campus managers	For visible leadership and support to the programme and many actions that facilitate its smooth operation at institution level.
HIV programme managers and communicators, campus health facility managers and staff, student liaison officers and student support service managers	For their daily actions to sustain and strengthen the prevention, care and treatment of HIV, TB, STIs and other health conditions.
HIV Institutional Coordinating Committees	For leadership and lobbying for the HEAIDS cause and guidance to programme implementation teams.
Student leadership	For devoting time to HEAIDS in an often-difficult climate and seeing its intrinsic value to students.
Peer education mentors and peer educators	For contributing to the building of a sustainable HIV, TB and STI programme and giving it an authentically young character.
Academic staff participating in the programme	For active support in the development and delivery of curriculum content that contributes to our capacity to respond to these major health threats.
Campus and community radio stations in the Future Beats initiative	For taking seriously your mandate to serve and educate and improving health awareness among your listeners.

Finally, I would like to pay tribute to my team in the HEAIDS National and Provincial Office. Advocating, organising, enabling and coordinating are the essence of their daily work. This kind of activity requires resilience, diplomacy, confidence and good judgment – and the rewards are long-term rather than immediate. In other words, HEAIDS requires special qualities in its people and I appreciate the contribution that our staff – with their diverse attributes and approaches – make to the organisation.



Ramneek Ahluwalia
HEAIDS Director
Programme Director

3



HEAIDS Programme Performance

3.1 Universities Programme

Through its Universities Programme, HEAIDS has continued to support the country's 26 public universities in the planning and delivery of their HIV interventions and other health promotion initiatives.

Like most other aspects of university life, HIV programmes were affected by the intermittent interruptions caused by fees protests and the uncertain climate that prevailed during these times. Therefore, it was no small achievement that HIV programmes almost held the ground gained in 2015, when very large increases in activations and HIV testing were recorded.

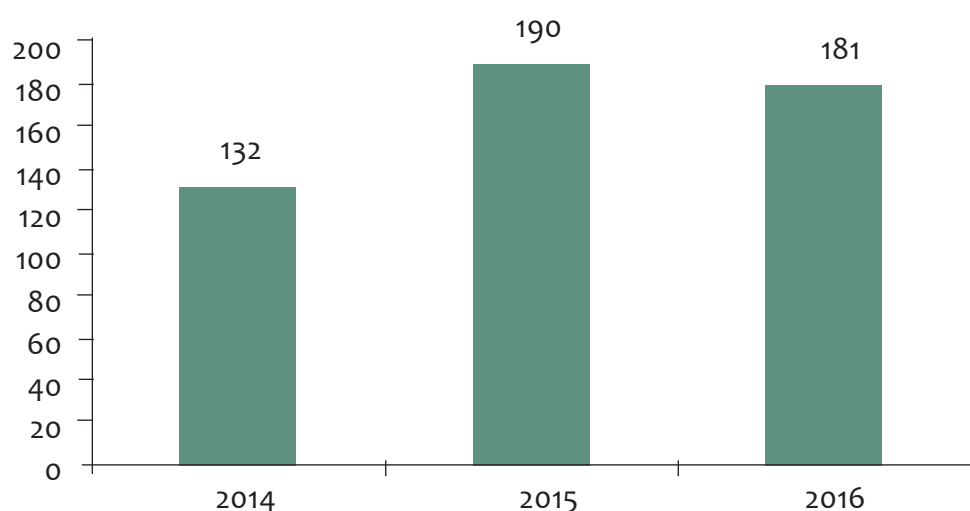
First Things First activations

The main vehicle for HEAIDS in the areas of health promotion and early intervention is an annual series of First Things First (FTF) activations on campuses across the country. Each activation unfolds over approximately three days, combining awareness-raising and informational activities with screening for HIV and other health conditions.

Although prevention of HIV, TB and STIs remains at the heart of all activations, HEAIDS encourages universities to approach sexual and reproductive health needs more holistically during these activities and to incorporate the prevention of non-communicable diseases. The activation model is sufficiently flexible to take on themes that are particularly relevant at the time and address particular priorities of individual institutions.

Both the number of FTF activations undertaken on university campuses and the total number of individuals tested for HIV were slightly lower in 2016 than in 2015. The number of activations dropped by nearly 5% to 181, while the number of HIV tests performed was about 8% lower, reaching about 81 000 students and staff members.

Figure 6: Number of FTF activations at university campuses 2014 - 2016



The fact that a number of universities were able to organise more than one activation during 2016 helped to sustain the level of performance. HEAIDS regards a minimum of two activations a year as ideal because this increases students' access to testing and screening.

Although universities in Gauteng, KwaZulu-Natal and the Western Cape experienced repeated or sustained disruption of classes due to protests, they still made the strongest contributions to activations and HIV testing.

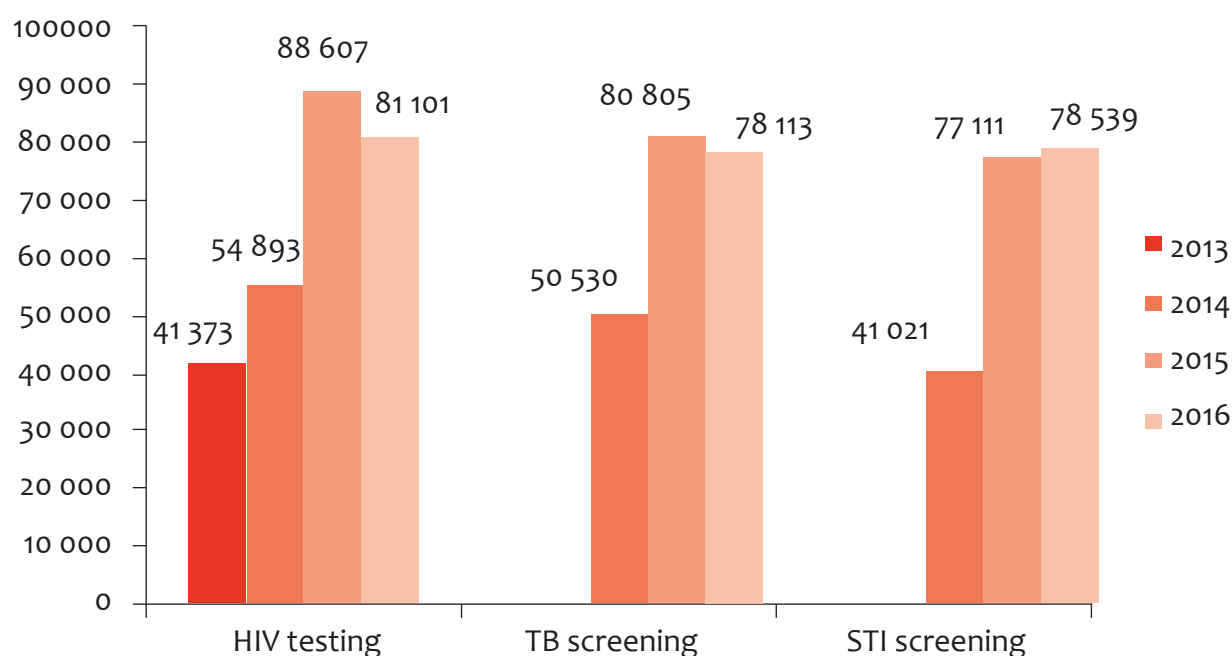
In some provinces, factors besides campus instability impacted on the university sector's ability to organise FTF activations. Some organisations that had served as crucial partners in delivering FTF were unable to sustain their participation in 2016. This was due to changes in the funding that they received through international development assistance.

The Department of Health and provincial health departments stepped forward to facilitate new partnerships with non-governmental organisations that they support. In many cases this enabled campus activation to proceed, but there were instances where workable alternatives could not be found within the time available.

HIV testing and other health screening uptake

HIV testing is virtually always accompanied by TB screening and STI screening during FTF activations. The moderate drop in the number of HIV tests performed in 2016 relative to the previous year has already been noted. In contrast, the number of individuals screened for STIs actually increased, indicating that health workers used the opportunities presented by HIV testing very effectively to screen for other STIs.

Figure 7: Numbers tested for HIV and screened for TB and STIs at universities 2013 - 2016

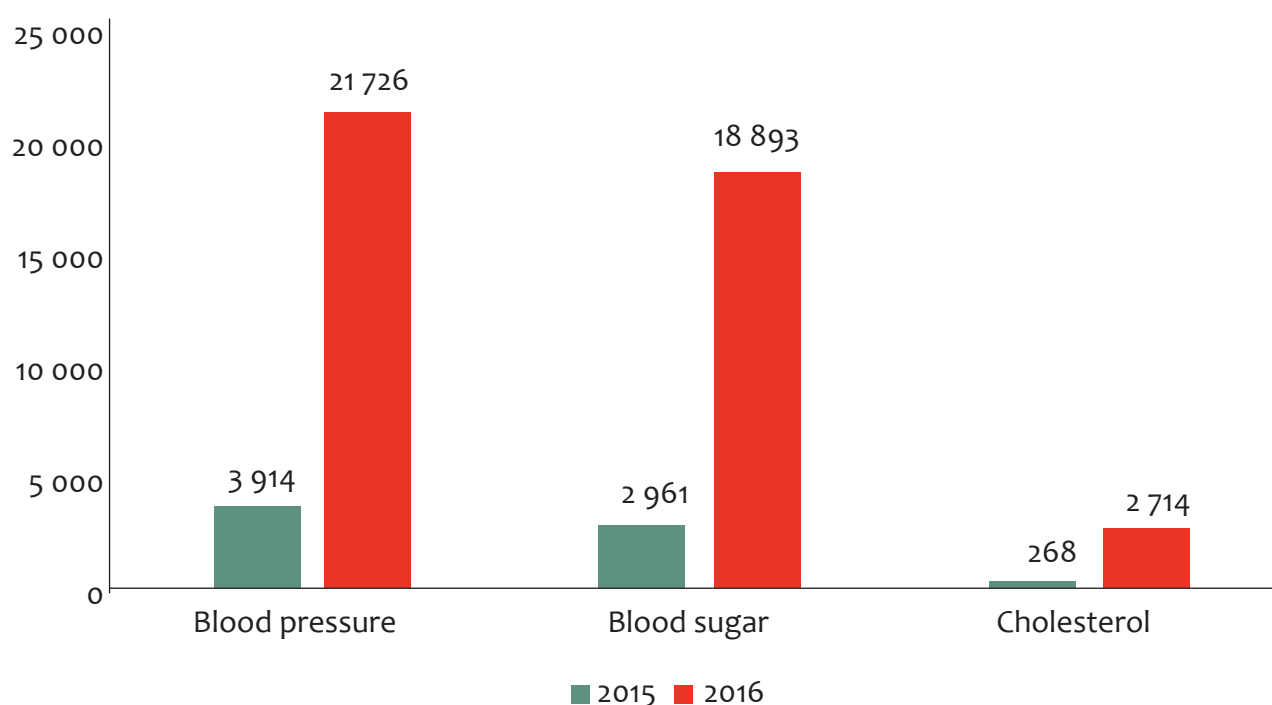


HEAIDS has envisaged for some time that FTF would evolve into a comprehensive preventive health campaign, balancing its focus on the major communicable diseases with a strong non-communicable disease component and expanding the scope of its sexual health services. Real progress was made in this respect in 2016:

- Blood pressure measurements were taken for 21 726 students and staff members.
- Blood sugar checks performed on 18 893.
- Contraceptives were provided to 14 203 during activations and through campus health clinics.

In addition, universities reported they had referred 299 men for medical circumcision, while some institutions provided screening for breast cancer and cervical cancer. The reported numbers are still relatively low but they speak to the possibility of achieving more holistic preventive health care on campuses.

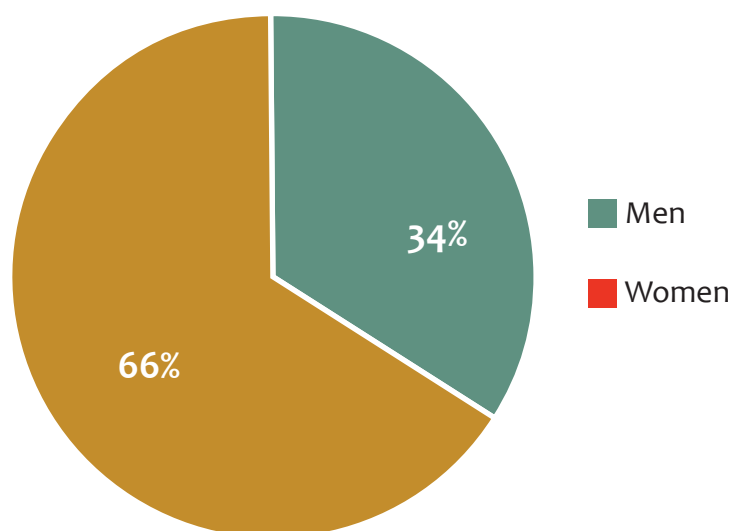
Figure 8: Number of individuals screened for non-communicable diseases 2015 - 2016



Universities also provided post-exposure prophylaxis (PEP) to prevent possible HIV infection in 288 instances. The circumstances in which PEP was provided are not known. Formally, PEP is available as part of the package of care provided after sexual assault and also after occupational exposure to HIV among healthcare workers, including students in the health professions. However, it is sometimes provided after unprotected consensual sex in circumstances where there is a high risk of HIV infection.

One in four individuals who tested at universities in 2016 was a first-time tester, so FTF clearly provides an enabling environment for many people. There is still a strong predominance of women among those using campus testing services: two out of three students and staff members undergoing testing are women. This suggests a greater sense of personal risk of infection among young women – which is in line with the gendered reality of the HIV epidemic in South Africa.

Figure 9: Uptake of HIV testing services at universities by sex, 2016



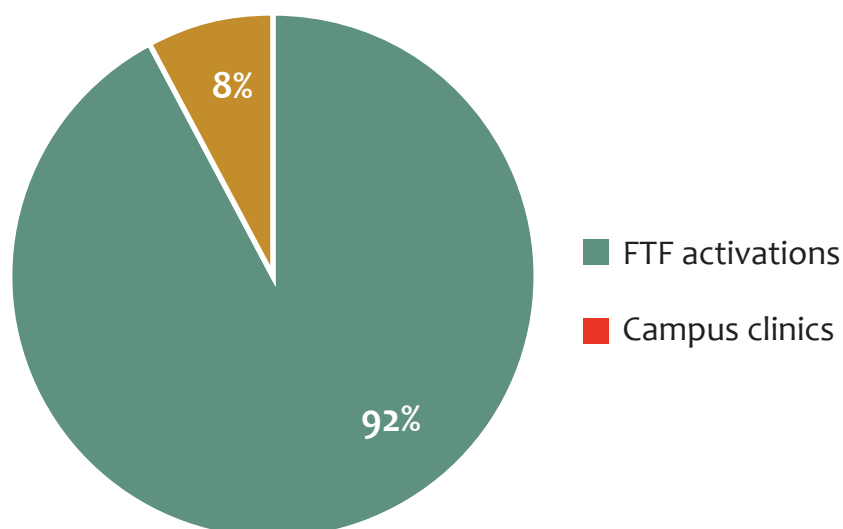
Although the HIV risk to young men in South Africa is not as great as that faced by young women, it is nevertheless substantial. Therefore, HEAIDS utilises its Men's Health Programme to encourage men to know their status. In 2016 universities began to integrate sports activities with screening services for men and this may account for the small increase in uptake that occurred among men during the year.

Peer education training

More than nine out of 10 individuals who took an HIV test on a university campus in 2016 did so in response to FTF activations, while the remainder tested at campus health clinics. These numbers indicate a strong preference for initiatives driven by peer educators which create an inviting, vibrant environment that encourages students to step forward and get tested.

In addition, at least 163 431 students and staff participated in other peer educator-driven activities, including dialogues and campus campaigns.

Figure 10: HIV testing option preferred by university students and staff 2016



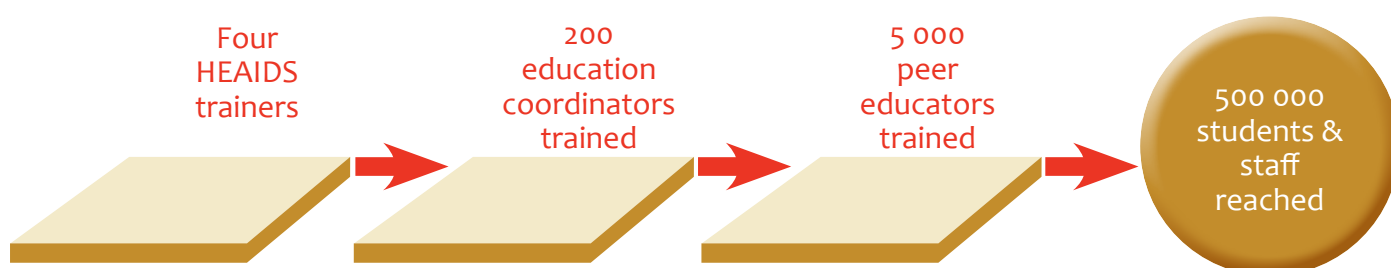
HEAIDS encourages universities to build strongly on the success of peer-to-peer interventions, while at the same time maximising the potential of campus health services to contribute to HIV prevention through provider-initiated counselling and testing (PICT). The latter involves the proactive offer of an HIV test to every individual who utilises the health service, regardless of the initial reason for the consultation.

Peer education training went from strength to strength in 2016. The operational capacity of campus HIV programmes depends to a large extent on having welltrained and highly motivated peer educators available to interact with students.

During 2016, HEAIDS used a combination of training models in order to maximise its capacity to prepare volunteers for their role in HIV programmes. This resulted in the training of a record number of university peer educators – a total of 1 875, which represents a 21% increase on the previous year's figure. While some of these peer educators graduated at the end of 2016, a high proportion will still be on campus for at least another year.

The HEAIDS strategy for strengthening the peer education system in the future is to build a stable core in the form of 200 peer education coordinators at institutional level. They will ensure continuity from year to year and renewal of the system as peer educators graduate and need to be replaced. It is envisaged that the new system would be capable of reaching out to 500 000 students and staff members each year – that is, about half of the national public university population.

Figure 11: Model for scale-up and sustainability of peer education system at universities



Condom distribution

During 2016 a total of 14 412 641 male condoms and 106 357 female condoms were distributed at universities. These are all government-issue condoms and universities have benefited from the expansion of the range to include various coloured and fruit-scented condoms and the rebranding of the product range as the MAX condom. A total of 69 730 tubes of lubricant were also made available.

Institutions are supplied from provincial health department depots and the availability of stock varies from province to province. In some instances, universities could distribute more condoms if supplies were available. Part of the educational role of peer educators is to share information on correct condom use.

Advocacy, coordination and support

The HEAIDS programme office interacts with a wide range of individuals and organisations, both within the university sector and beyond, in order to sustain and enrich the campus HIV response. The success of HEAIDS' work depends critically on the contributions of a wide range of parties and we devote considerable time and effort to knitting these contributions into a single fabric. The programme office achieves this by means of:

- High-level meetings with university vice-chancellors and the HIV, health and wellness coordinating committees at individual institutions.
- Visits to universities for purposes of technical capacity-building.
- Regional workshops, which not only involve universities in the area but also critical partners for HIV programme development, both in the public and non-governmental sectors.
- Meetings with the executive committees of national forums for university HIV programme managers, heads of campus health services, and representatives of employee health and wellness programmes at universities.

Each year, as the wider environment changes, we find new issues emerge and there may be a need to advocate modifications to our shared strategic approach to HIV and wellness in the sector.

During 2016, student protests led to the termination of many outsourcing contracts at universities across the country and the inclusion of large numbers of unskilled and semi-skilled service workers on their staff establishments. The HIV prevalence survey conducted in the sector several years ago showed a higher prevalence of HIV among service workers than other categories of university staff. We are encouraging universities to consider how their employee health and wellness programmes could best respond to the needs of newly incorporated service workers.

3.2 TVET College Programme

The TVET college sector – comprising 50 colleges and 244 campuses – has unique requirements as it develops its capacity to respond to HIV and other relevant health conditions among its students and staff members. During 2016, HEAIDS strove to increase its support to colleges as it gained a deeper understanding of their needs.

The HEAIDS strategic approach was established essentially with the participation of public universities before the inclusion of TVET colleges in our programme. This strategic framework is certainly relevant to TVET colleges, but important modifications have been necessary at the operational level.

It was recognised at the outset that HEAIDS would provide more direct support to TVET colleges than to universities, because very few colleges have staff dedicated to HIV prevention or on-campus health facilities. Provincial coordinators employed by HEAIDS have been at the centre of the TVET College Programme from its inception.

In 2016 HEAIDS took important steps to strengthen its network of provincial coordinators – who had become stretched beyond effective limits – and prioritised discussions with health departments and other potential partners whose support would enable TVETs to offer quality health activations to their students and staff members.

First Things First activations

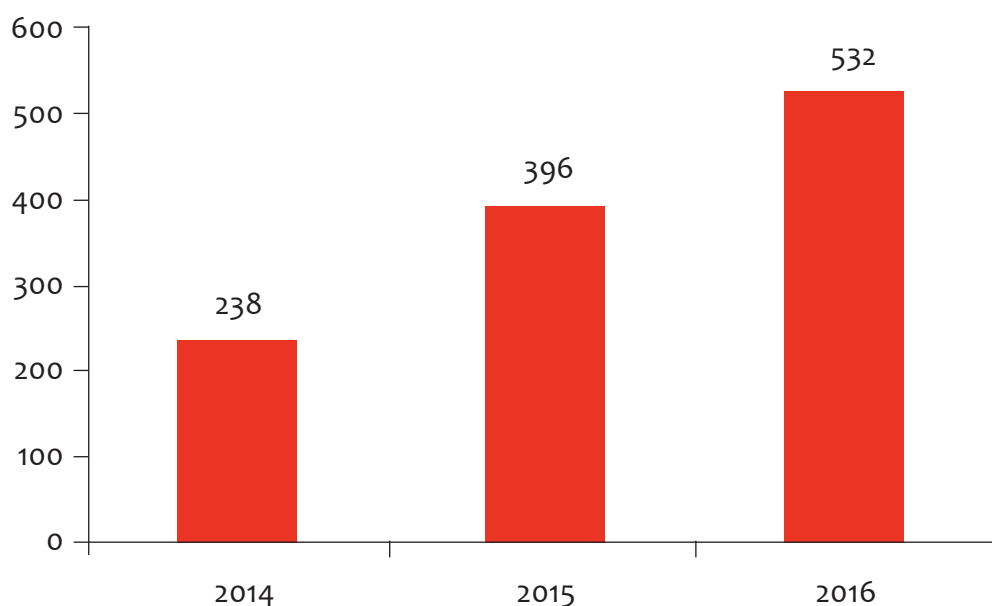
The character of FTF activations at TVET colleges is essentially the same as those organised at universities, with each activation featuring a series of campus-based awareness-raising, educational and screening activities over a period of about three days.

The number of activations undertaken by TVET colleges increased substantially in 2016, totalling 532. This represented a one-third increase on the 2015 figure and the fact that some colleges were disrupted by fees-related protests makes this this effort all the more impressive.

At all colleges, external support is vital to the organisation of high-quality activations. HEAIDS provincial coordinators strengthen the hand of college personnel in forging partnerships with organisations that can assist. Coordinators are present at each college in the immediate build-up to each activation or during the event.

Trained peer educators at TVET colleges are critical to the marketing of activations and responsible for many aspects of health promotion.

Figure 12: Number of activations at TVET colleges 2014 - 2016



In the TVET College Programme, as in the University Programme, the process of expanding the focus of activations to address sexual health more broadly and prevention of non-communicable diseases has begun on a modest scale.

Provincial activations

Each year, some FTF activations are selected as provincial activations, where the participation of national and provincial education and health sector leadership attracts important stakeholders and local media. Provincial activations serve as a platform for advocacy, both for the HEAIDS programme and for specific health goals.

Provincial activations in 2016 are described in Table 2.

Table 2: Provincial activations held at TVET colleges in 2016

College	Keynote speaker
Northern Cape Urban TVET College: Moremogolo Campus (1 March 2016)	Deputy Minister of Higher Education and Training Mduzuzi Manana
Ehlanzeni TVET College: Mlumatl Campus (29 April 2016)	Deputy Minister of Higher Education and Training Mduzuzi Manana
Sekhukhune TVET College: CS Barlow Campus (17 May 2016)	Limpopo MEC for Health Dr Phophi Ramathuba
Thekwini TVET College: Melbourne Campus (20 June 2016)	KwaZulu-Natal MEC for Health Dr Sibongiseni Dhlomo
Lovedale TVET College: Zwelitsha Campus (14 November 2016)	Deputy Minister of Higher Education and Training Mduzuzi Manana

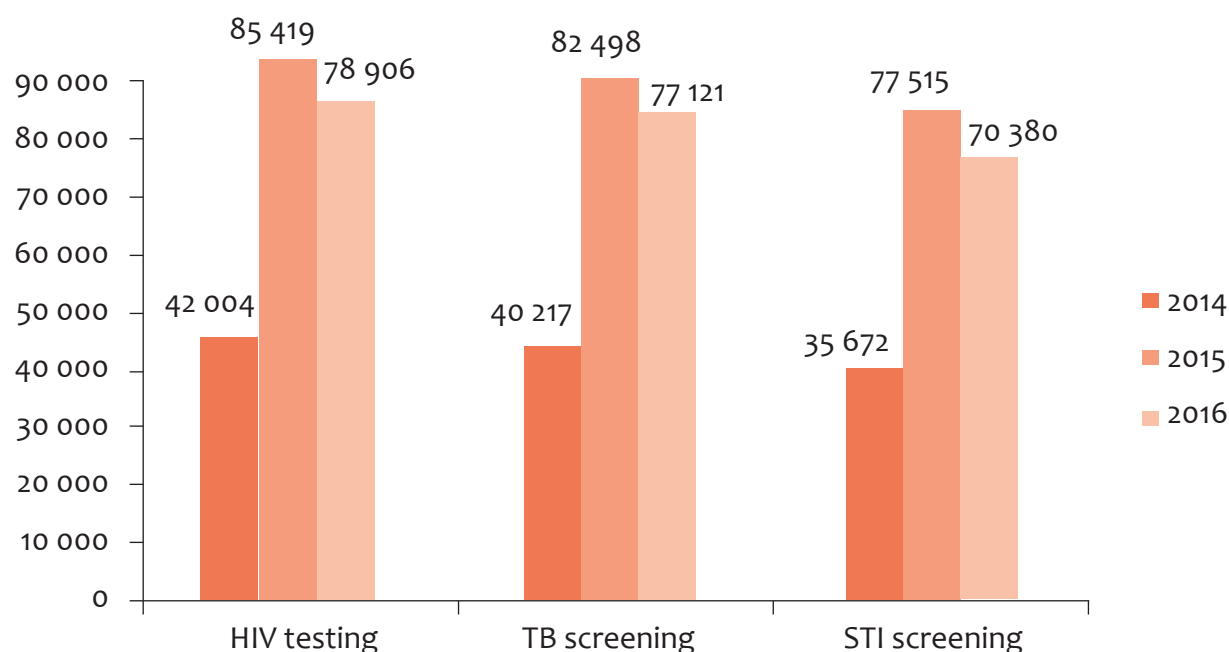
Uptake of HIV testing and other preventive health interventions

Despite the upswing in the number of activations, there was a slight drop in the number of individuals who tested for HIV and underwent screening for TB and STIs. However, the numbers were still substantial, with nearly 79 000 HIV tests performed and almost as many TB screenings. The levelling-off in testing figures has prompted HEAIDS to reflect on how to improve service uptake and get maximum value from every activation.

Even before the year began it was clear that HEAIDS' provincial coordinators could not keep pace with the demand for activations. At the start of 2016, HEAIDS had only eight coordinators and it was estimated that some 488 activations would be required during the course of the year. This would yield an average workload of 61 activations per coordinator per year – or more than one a week. Clearly, this would not allow them to ensure that there was careful preparation and sound coordination of all activations.

HEAIDS sought and received additional funding to expand its team of provincial coordinators. This allowed for the engagement of an additional five coordinators as from July 2016 and the allocation of two coordinators apiece to Gauteng and KwaZulu-Natal, where the highest number of colleges are situated. The benefits of this expansion will be felt more strongly in 2017.

Figure 13: Numbers tested for HIV and screened for TB and STIs at colleges 2014 – 2016



Through partner participation, TVET colleges were able to screen quite large numbers of students and staff for high blood pressure (21 451) and high blood sugar (16 476). Smaller numbers were screened for cervical and breast cancer, while 576 were referred for medical male circumcision (MMC) and 1 301 provided with contraceptives of their choice.

Condom distribution

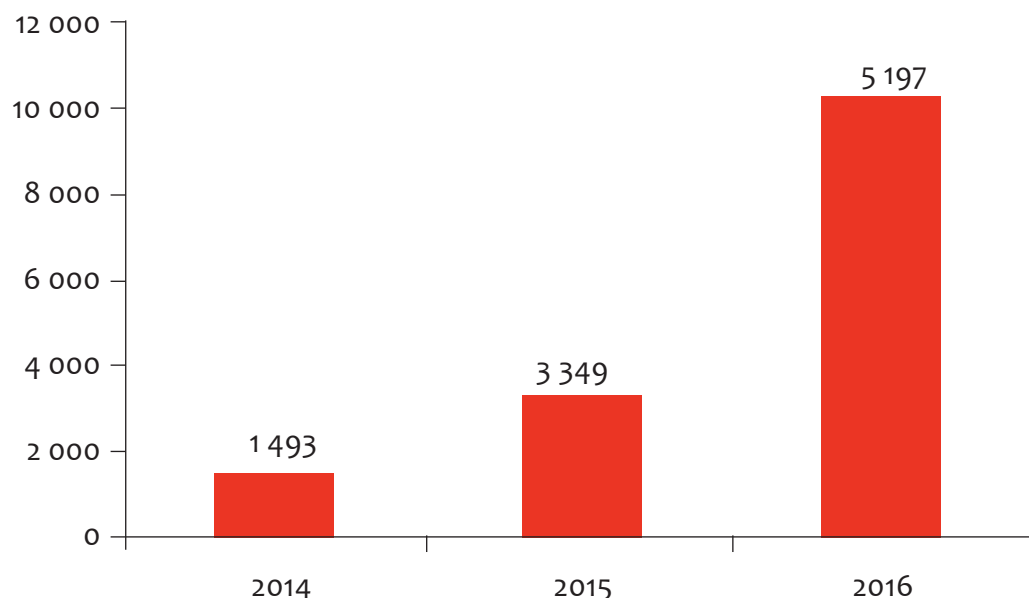
Condom distribution sounds like a simple activity, but it requires some infrastructure in order to maintain supplies. The reliability of the distribution system is critical in the light of the emphasis placed on consistent – rather than occasional – condom use.

At TVET colleges, HEAIDS was responsible for distributing a modest number of condoms in 2016, totalling just 1.36 million – a fraction of the number supplied to universities. A priority for 2017 is the organisation of supply systems, the placement of condom dispensers at appropriate points, and the incorporation of education in correct condom use into the activities of peer educators.

Training of peer educators

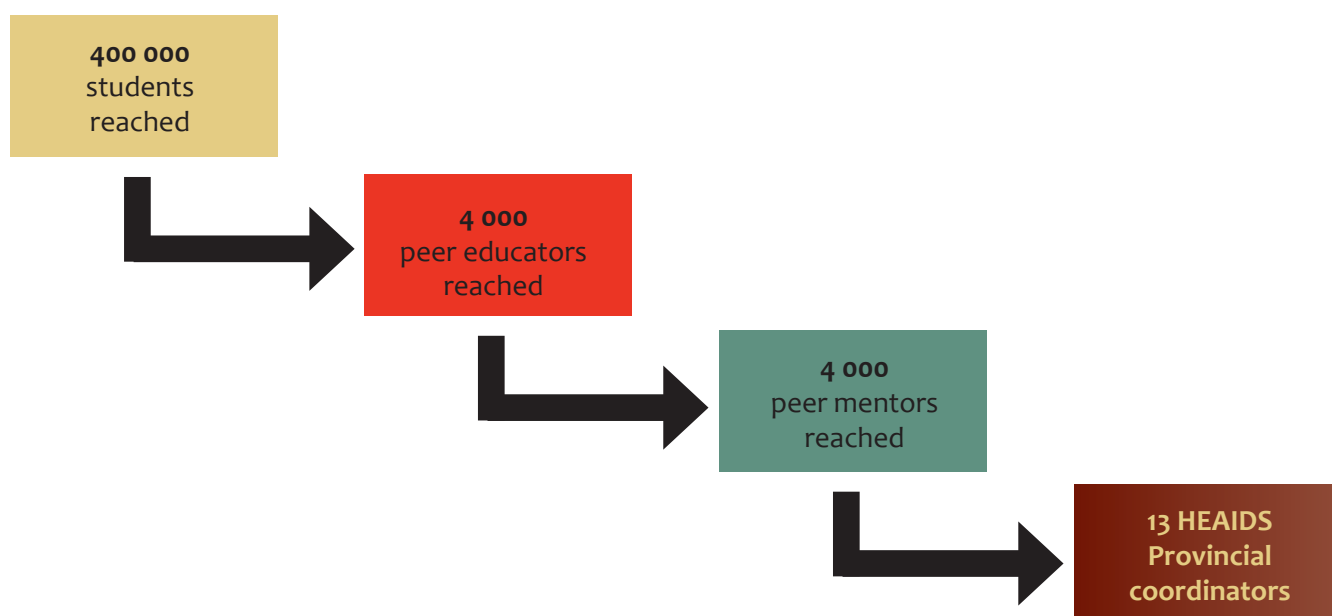
In 2016, the number of college-based peer educators who underwent training was more than triple the number trained in the previous year and it exceeded the target by a wide margin. Nearly 10 500 peer educators received training as a result of an enthusiastic response from the colleges and HEAIDS' determination to maximise training opportunities.

Figure 14: Number of college students and staff trained in peer education 2014 – 2016



While 2016 yielded a bumper crop of peer educators, HEAIDS has developed a model for a sustainable peer education programme and the targets going forward will be more moderate. This model is depicted in Figure 15. It depends critically on the establishment of a core group of peer mentors who are drawn from the staff of TVET colleges and provide continuity to the programme as successive waves of peer educators are recruited, trained, and deployed in campaigns until the point of their graduation.

Figure 15: HEAIDS peer education model for TVET colleges



Mentors not only coordinate peer activities at individual colleges but are also equipped to provide training to peer educators. This train-the-trainer model is a sustainable approach but regular refresher training will be needed to keep mentors abreast of the latest developments in the field of HIV prevention and consequent adjustments to the HEAIDS programme.

Coordination, support and advocacy

In 2016 provincial coordinators arranged and participated in 661 meetings with representatives of TVET colleges and other stakeholders involved in the delivery of the HEAIDS programme at colleges. About eight out of 10 visits were purely with college staff and students.

Three provinces – Gauteng, the Western Cape and KwaZulu-Natal – accounted for about 60% of coordinating meetings. This is understandable, as the large urban centres in these provinces contain numerous colleges and campuses.

The development of structures at college level to sustain the HIV- and health-related interventions on campus saw major gains in 2016. By the end of the year 90% of colleges had set up HEAIDS institutional coordinating committees and these provided a solid centre for the programme at campus level.

3.3 HIV Curriculum Development Programme

The Curriculum Development Programme strives to give effect to the HEAIDS objective of harnessing the higher education sector's contribution to the country's knowledge economy in order to strengthen the national HIV response. The effect of this programme should be apparent in graduates' personal and professional competence to manage HIV as it impacts on their lives at home, in their communities and places of work.

In order to have this impact, the programme develops the knowledge and skills of academic staff at universities and TVET colleges to enable them to integrate HIV-related issues into the curricula they offer and encourages the development of relevant new curricula.

Key activities undertaken during 2016 were:

- Management of direct grants made to 20 universities by the National Skills Fund (NSF) for the purposes of curriculum development and support.
- Mobilisation of new funds for the next phase of the programme.
- Facilitating national capacity development workshops for academic staff.
- Advocating for HIV curriculum development within the sector.
- Coordinating research for the sector in the teaching and learning space.
- Taking forward the development the HIV curriculum in three TVET work streams, life orientation, agriculture and mining.
- Monitoring and evaluating the programme with a view to establishing its impact on academic staff and their students.

The curriculum development programme has gained significant momentum in the university and college environments and, as the initial grant ends in 2017, mobilising new funding has become a priority to extend the progress made.

Universities and HIV curriculum development

In securing direct grants from the National Skills Fund for 20 of the 26 universities in the sector, HEAIDS has enabled universities to contribute to the body of knowledge on HIV curriculum development and to the relevance of curricula and their role in social transformation.

In keeping with the core objective of capacitating academics, five thematic areas have been identified and workshops are offered in these areas. A total of 35 workshops have been conducted in about two years and have been attended by approximately 850 academic and support staff. In this manner, the sector has established a growing pool of key experts in the area of HIV curriculum development.

The workshop in critical diversity literacy has gained momentum across the sector. It links into the transformation agenda that pervades the teaching and learning space at our universities, speaking to issues of race, power and privilege. The workshop draws on the work of Melissa Steyn and seeks to engage academic staff in the analysis of unequal power relations. It suggests that inequity is reinforced by cultural practices and institutions and aims to help staff develop tools to challenge this inequity.

TVET colleges and curriculum development

The NSF grant has also supported activities in the TVET sector and this benefits all 50 colleges. Three key work streams are the focus of curriculum integration in the college sector – life orientation, agriculture, and mining and engineering. The selection of these work streams was informed by a baseline study undertaken to better understand the teaching and learning environment in TVET colleges.

In relation to the life orientation work stream, teaching of HIV-related content is now guided by assessment guidelines which were introduced in 2016. These were developed in order to enable lecturers to strengthen their teaching of HIV and related topics. An assessment workbook was developed and tested with focus groups in three provinces before being introduced nationally. Understanding of the approach was promoted through ongoing HEAIDS interaction with life orientation lecturers.

Higher education institutions face demands from many quarters to expand their teaching curricula. In the light of this, HEAIDS undertook extensive communication to college principals and senior staff members to win support for the curriculum-based approach to addressing HIV. These activities included a presentation at a principals' meeting hosted by the Department of Higher Education and Training (DHET) in September and participation in the Pan African TVET and FET colleges' conference in Cape Town in October.

Other achievements for the year were:

- The development of a theory of change which provides structure and direction for curriculum-level interventions.
- The creation of a robust monitoring and evaluation framework which will be used in 2017 at both universities and colleges to measure the impact of the programme.
- The establishment of a community of practice that links various institutions and allows participants to engage regularly with each other and share experience.
- Significant building of capacity at participating institutions through HEAIDS interaction with institution-based coordinators for curriculum development.

3.4 Future Beats Youth Development Programme

Future Beats is a youth development and HIV prevention programme that aims to enhance campus and community radio stations and the use of social media in order to increase young people's awareness of HIV and related social and human rights issues.

The project empowers programme managers and volunteer journalists at campus and community radio stations to produce interesting, accessible content on these subjects and engage in dialogue with their audiences. The programme is funded by the German Development Agency, GIZ.

Future Beats began in 2014 and has grown steadily to encompass more radio stations. In 2016, the programme had 14 participating radio stations with a combined listenership of about 720 000. A formal evaluation undertaken at the end of the first year of operation indicated that the programme not only achieved the upskilling of radio staff and focused them on HIV-related issues, but was also associated with an increase in HIV testing on three campuses.

The participating radio stations during 2016 were:

- Alfred Nzo Community Radio – Mount Ayliff, Eastern Cape.
- Barberton Community Radio – Barberton, Mpumalanga.
- CUT Radio – Central University of Technology, Bloemfontein, Free State.
- Emalahleni FM – Emalahleni, Mpumalanga.
- Forte FM – University of Fort Hare, Alice, Eastern Cape.
- Kopsie FM – University of the Free State, Bloemfontein, Free State.
- Lukhanji FM – Queenstown, Eastern Cape.
- Madibaz Radio – Nelson Mandela Metropolitan University, Port Elizabeth, Eastern Cape.
- Maputaland Radio – Jozini, KwaZulu-Natal.
- PUK FM – North West University, Potchefstroom, North West.
- Radio DUT – Durban University of Technology, Durban, KwaZulu-Natal.
- Rhodes Music Radio – Rhodes University, Grahamstown, Eastern Cape.
- UNISA Radio – University of South Africa, Pretoria, Gauteng.
- VOW FM – Wits University, Johannesburg, Gauteng.

Figure 16: Expansion of radio stations participating in Future Beats Programme 2014 - 2016



Future Beats capacity development

A total of 84 campus and community radio journalists benefited from a series of orientation and training workshops held during the course of 2016. These workshops are summarised in the table below.

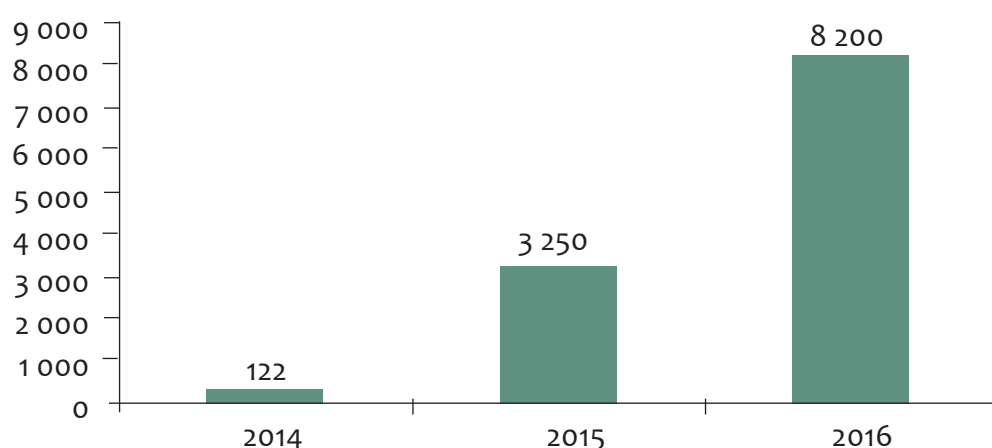
Table 3: Summary of Future Beats workshops/training sessions held in 2016

Type of workshop or training session	Number held and length	Participants	Partner/guest presenter
Planning and networking workshop to introduce new stations to Future Beats project design	One three-day workshop	New radio stations	N/A
HIV sensitisation training to highlight HIV transmission and social drivers of the pandemic	Three two-day workshops	New radio stations	Applied drama facilitator and HIV expert Tshego Khutsoane
In-house training on journalism techniques for production of high-quality content on HIV and related issues	Seven three-day training sessions	New radio stations	Executive Producer of Wits Radio Academy Xoli Matomela
Facilitation training for presenters, focusing on techniques for sensitive topics. Designed to support stigma reduction	One two-day training session	Presenters at Maputaland Community Radio, KZN	Radio producer and facilitation trainer Lesley Nkosi
National workshop to share best practices of HIV reporting, plus additional sensitisation training on gender-based violence	One two-day workshop	All current partner stations of the Future Beats project	CEO of Gender Links Colleen Lowe Morna

HEAIDS social media

Future Beats has benefited HEAIDS more generally by contributing to the development of the organisation's social media platforms. During 2016, HEAIDS had a presence on Facebook, Twitter, YouTube and Soundcloud and its social media followers totalled about 8 200 in December 2016.

Figure 17: Growth in HEAIDS Facebook followers 2014 – 2016



3.5 Research Projects

A constant feature of HEAIDS' work is the generation of evidence for the planning of interventions and evaluation of the impact of various programmes. During 2016 the two major items of research were:

- An impact evaluation of the FTF programme, which was undertaken by the Human Sciences Research Council on behalf of HEAIDS.
- An evaluation of the impact of the Future Beats radio and social media programme on the knowledge, attitudes and practices of its audiences.

The impact evaluation of the **First Things First programme** was undertaken in 2015 and published in 2016. It aimed to establish:

- The relevance of FTF – that is, how well suited it was to the wider objectives of HEAIDS and to the HIV priorities of the higher education and training sector.
- The impact of FTF in terms of changes produced.
- The efficiency of the programme in terms of providing timely services of a satisfactory standard.
- The effectiveness of the campaign in terms of making a contribution in proportion to the investment entailed.
- Programme sustainability, which was gauged by the additional resources that were leveraged to prolong its beneficial effects.

The study included a value-for-money evaluation, based on various documents relating to the FTF programme, and qualitative research which encompassed interviews and focus groups with stakeholders. The average rating awarded for the value-for-money component was 71% while the qualitative process was converted to an average score of 86%.

The researchers concluded that FTF had made a very positive impact in the higher education environment and was considered as relevant to the HIV and health challenges in this sector. Both the buy-in and support of leadership at universities and colleges and the availability of funding through the Global Fund grant were noted as critical success factors.

The recommendations made as a result of the evaluation included the following:

- Networks and relationships linking HEAIDS, universities and colleges, funding organisations, service partners and other stakeholders were the lifeblood of FTF and must be sustained in order to ensure that FTF remains relevant.
- Stronger oversight by HEAIDS of campaign activities was necessary to ensure that counselling and testing complied with high standards wherever FTF took place and that data was submitted on time.
- Use of structured M&E tools was required to measure the progress and ultimate impact of the FTF approach in terms of its objectives.
- Monitoring of the demand for FTF and emerging needs on various campuses should be undertaken in order for the programme to respond accordingly. In addition, the principle of equity should be applied in terms of extending the campaign to all campuses of universities and colleges.
- Measures should be put in place to ensure the sustainability FTF because of its intrinsic value to campus communities.

The 2016 evaluation of **Future Beats** from the perspective of audience response complemented an earlier evaluation which examined the value the project represented to participating radio stations. The research was undertaken by Initiative International. It examined changes in relation to knowledge, attitudes and behaviour related to HIV over a period of six months among 2 000 students who listened to four radio stations in the Future Beats programme: Barberton FM, Emalahleni FM, Forte FM and Rhodes Music Radio.

Preliminary results show that Future Beats stimulated discussion among friends about sexual subjects that are usually avoided and increased uptake of relevant services at campus and community health facilities.

3.6 Communication Activities

The HEAIDS programme office is essentially a catalyst and enabler of HIV-related interventions that are undertaken by individual universities and TVET colleges, supported by a range of partners from civil society and government. Communication is a critical factor in ensuring that all organisations in this complex system understand the objectives of the HEAIDS programme and are able to mobilise their students and staff to participate in its activities.

The face-to-face communication initiated by the teams responsible for the University and TVET College Programmes and Future Beats are complemented by the proactive use of the mass media to raise the profile of the HEAIDS initiative.

Mass media profiling of HEAIDS

In 2016, at least 151 news items on HEAIDS appeared on radio and television and in print and online media. This publicity was worth some R7.36 million, when valued at the cost of an equivalent volume of advertising.

About half of HEAIDS-related news items were carried in the broadcast media and the average value of each item was higher than that in 2015, reflecting the interest of several major broadcast media, such as Metro fm and the 24-hour news channel, eNCA.

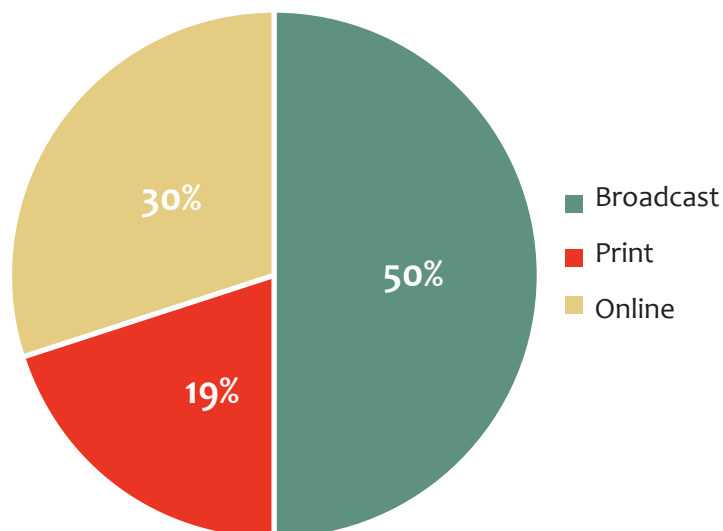
Media interventions took the form of:

- Proactive sharing of work undertaken by the HEAIDS programme – for example, the release of the TVET knowledge, attitudes and behaviour study.
- Responsiveness to issues related to the health of students – for example, the serious matter of sexual assault on campuses.

The presence of the Deputy Minister of Higher Education and Training at several FTF provincial activations attracted local media and the resulting coverage served to inform communities of developments at their local colleges.

HEAIDS Executive Director Dr Ahluwalia was invited to participate as a panel member in a Mail & Guardian Critical Thinking Forum on the transformation of South Africa's HIV treatment programme in the period 2000 to 2016. This took place shortly after the International AIDS Conference had been hosted in Durban and served to bring some of the discussions of the conference to a broader, less specialised audience in Johannesburg.

Figure 18: Composition of media coverage on HEAIDS in 2016



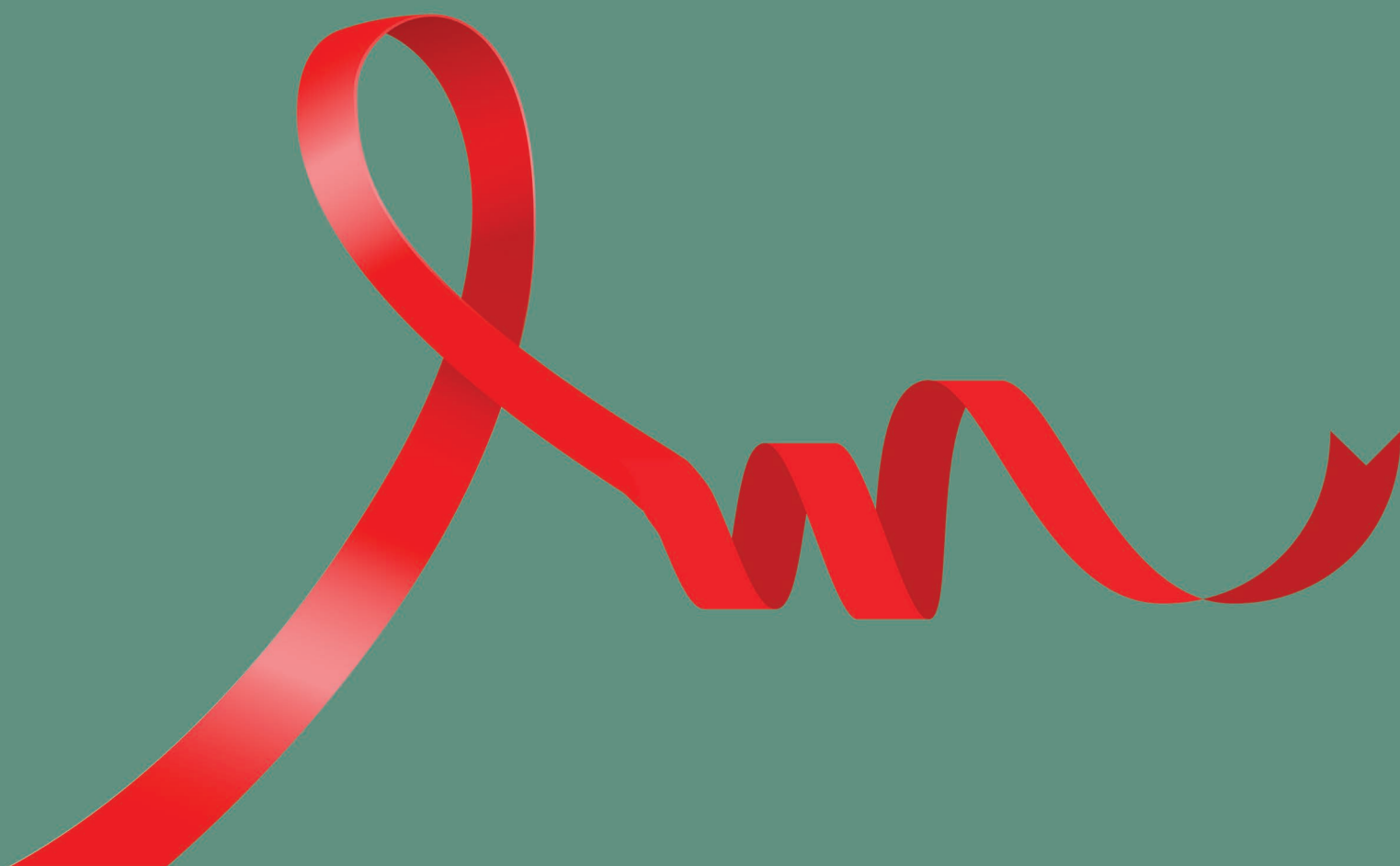
Publications and campaign branding and collateral

HEAIDS supports FTF campaigns and other events through the supply of branded collateral, ranging from banners and posters to informative leaflets and pamphlets.

At stakeholder events, research reports and programme reports were made available to participants. Publications produced during 2016 included:

- The 2015 Annual Review of the HEAIDS Programme.
- A manual entitled *Crucial Conversations for Peers: a fresh approach*.
- A leaflet on the peer education approach.

4



Financial Report for 2016

4.1 Income and expenditure

HEAIDS' income for 2016 was R42,4 million – 20 % higher than its 2015 income – and its expenditure amounted to about R30.47 million.

The expenditure figures in this report are preliminary amounts produced prior to auditing and minor adjustments in the audited reports are possible.

Income 2016

Source of income	Amount
DHET allocation	8 100 000
NSF grant: TVET support	8 522 212
NSF grant: curriculum development	8 733 184
Global Fund grant: managed by NACOSA	6 219 117
Global Fund grant: managed by AFSA	6 092 492
University membership fees	1 500 000
TVET subscription fees	1 500 000
Blue Label	978 500
GIZ grant for Future Beats programme	800 000
Total	42 445 505

Expenditure 2016

Cost centre	Budget allocation	Actual spending
Programme implementation	R26 022 320	R16 058 088
Human resources	R13 758 084	R12 695 975
Administration	R2 665 101	R1 717 297
Total	42 455 505	R30 471 361

Total surplus*	R11 984 144
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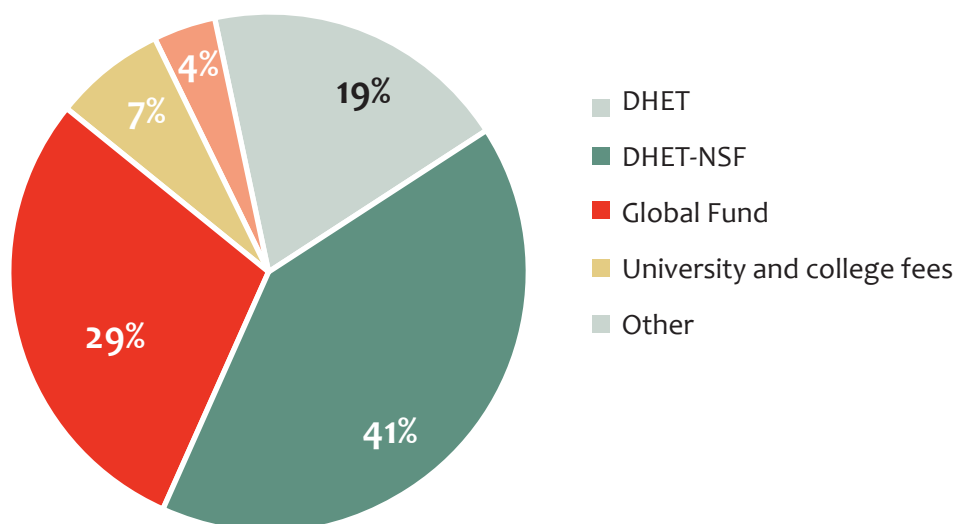
*It should be noted that some grants to HEAIDS correspond to the public sector financial year which ends on 31 March. They were therefore intended to fund activities up to that date.

4.2 Observations on sources of income

Domestic public sector funding of HEAIDS continued to increase, with a total of 68% of the budget derived from the DHET, the NSF and contributions from public universities and TVET colleges. This trend bodes well for the long-term sustainability of HEAIDS. However, in the current context international development partners still play a critical role in funding various programmes.

The diversity of funding sources is reflected in Figure 19.

Figure 19: Composition of HEAIDS 2016 income by source of funds



4.3 Patterns of expenditure

Spending on programme implementation absorbed 53% of expenditure in 2016, while human resources spending amounted to 42%. In 2015, human resources spending accounted for less than a third of total expenditure and the increased share of spending on this item in 2016 is reflected in the growth in the number of provincial coordinators.

Spending against the budget varied considerably by cost centre. While 92% of the human resources budget was utilised, only 62% of the amount earmarked for programme implementation was utilised.

- As indicated earlier, some of the unspent programme allocation is because government grants were intended to fund activities up to the end of March 2017.
- In the case of the Global Fund grant managed by NACOSA, the programme ended during the course of 2016 but some disbursements to universities could only be made in January 2017.
- In relation to the NSF grants there were some delays in implementation but plans were in place to compensate for this early in 2017.

Figure 20: Breakdown of HEAIDS 2016 expenditure by cost centre

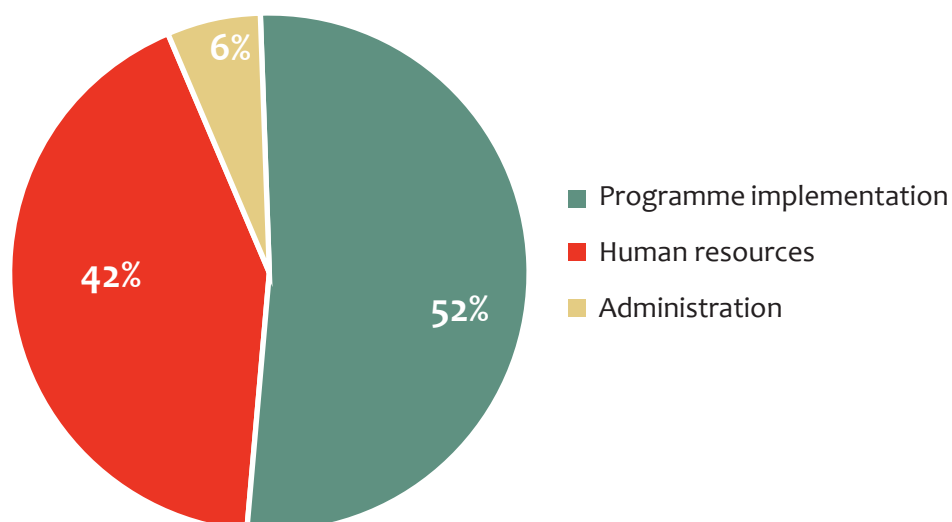
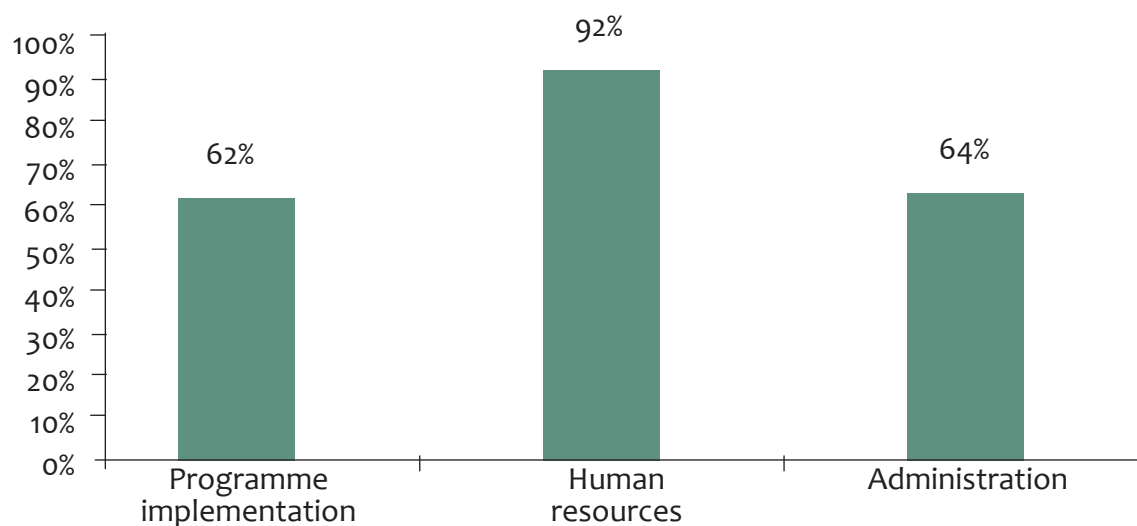


Figure 21: Percentage of 2016 budget utilised by cost centre



4.4 Auditing of HEAIDS programme

Fiduciary responsibility for the HEAIDS programme vests in Universities South Africa (USAf). Accordingly, the programme is audited as part of the USAf audit and an opinion expressed by the auditors on HEAIDS' financial management will be included in the broader audit report.



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